
Block 1: Concept of Development, Growth, Life Span Perspective, and Infancy

Unit 1: Concept of Development, Growth and Development, Life Span Perspective, Methods of Studying Development and Characteristics of Development

- **1.2 Concept of Development**
    - **Definition:** Development describes the **growth of humans throughout the lifespan, from conception to death**, encompassing patterns of change over time. It includes biological, physical, cognitive, and social aspects.
    - **Goals:** To **enable people to adapt to the environment** and achieve self-actualization. Lack of opportunities for self-expression can lead to frustration and negative attitudes.
    - **Nature of Change:** Development is a **continuous process**, though not constant; it occurs in waves, with competencies appearing, disappearing, and reappearing.
- **1.3 Growth and Development**
    - **Growth:** Defined as an **increase in size**. Refers to the development of children from birth to adolescence, often measured by weight and height against chronological age (growth curve).
    - **Development:** A **progression towards maturity**.
    - **Interaction:** Developmental change results from genetically-controlled **maturation** and environmental factors/learning, most commonly an **interaction between the two**.
    - **Factors Influencing Personality Development:**
        - **Society and Environment:** Strongest factors; children learn from surroundings and experiences.
        - **Cultural Factors:** Influence personality through music, television, modeling, and training.
        - **Place and Biological Factors:** Also involved as children learn to use body parts, express themselves, communicate, form relationships, care, love, and work.
    - **Four Areas of Growth and Development:**
        - **Physical:** Changes in height, weight, appearance (e.g., puberty), and physical abilities (crawling, walking, running, writing, refined motor coordination).
        - **Psychological and Cognitive:** Brain absorption of information, learning to think, process/organize information, solve problems, talk, complete mental tasks (e.g., remembering numbers).

        - **Social and Emotional:** Learning to interact, play, work, and live with others (family, friends, teachers, employers); understanding own and others' feelings; dealing with strong emotions; developing self-esteem, identity, and morality.
        - **Sexuality and Gender Identity:** Understanding body, what it means to be a boy/girl, differences between sexes, and responsibly handling sexuality throughout life.
- **1.3.1 Critical Period During Development:**
    - Specific periods where special stimulation is crucial for development.
    - **Example:** Children learn to trust parents through consistent love and care. Neglect and abuse can lead to distrust, which may transfer to other adults, though opportunities with foster parents can help build trust over time.
- **1.4 Life Span Perspectives**
    - **Definition:** The **ongoing process from conception to death**, involving methodical, intra-individual change associated with age and progressions in functioning. It involves examining both constancy and change in human behavior across the entire life span.
    - **Importance:** Helps describe and explain human development mysteries, applying knowledge to help people reach full potential.
    - **Multidisciplinary Study:** Human development is studied by psychology, sociology, education, and healthcare.
    - **Developmental Periods:** Childhood and adolescence, early adulthood, middle adulthood, and late adulthood. Transitions between these stages involve necessary changes in an individual's life character.
    - **Domains of Development:** Categories used by scientists, including **biological/physical**, **cognitive**, and **social/emotional**.
        - **Physical Domain:** How humans grow and change physically, impacting worldview.
        - **Psychological Domain:** Adjustment to the environment, essential for survival (e.g., sucking, eating, talking, walking, expressing); mastering adjustments to self, others, and environment becomes increasingly complex.
        - **Cognitive Domain:** Focuses on learning, attention, perception, memory, and school progress.
        - **Social Domain:** Adjustment to people, learning interactions, social skills, and developing relationships. All domains operate and affect each other.
    - **Four Interactive Forces Shaping Human Development:**
        - **Biological Forces:** Genetic and health-related factors (e.g., prenatal development, brain maturation, puberty, diet, exercise).
        - **Psychological Forces:** Internal perceptual, cognitive, emotional, and personality factors (e.g., intelligence, self-confidence, self-esteem).

        - **Socio-cultural Forces:** Interpersonal, societal, cultural, and ethical factors (e.g., family, peers, coworkers, social institutions, culture, poverty).
        - **Life Cycle Forces:** Differences in how the same event affects people of different ages; each individual is a unique product of these forces.
- **1.4.2 Issues in Life Span Development:**
    - **Continuity and Discontinuity:** Whether development is a smooth, gradual process (continuous) or marked by distinct, stable stages with qualitative changes (discontinuous).
    - **Stability and Change:** Whether behaviors or traits remain stable over time (e.g., shyness) or fluctuate across the lifespan.
    - **Nature vs. Nurture:** The debate over the relative contributions of heredity (genetics) and environment (learning) to an individual's behavior. Most psychologists believe it's an **interaction** between the two.
- **1.5 Characteristics of Life Span Development (Paul Baltes):**
    - **Lifelong Development:** Development potential extends across the entire life span; no age period dominates, and gains/losses occur throughout.
    - **Multidimensional:** Development cannot be described by a single criterion; it occurs in biological, cognitive, and socio-emotional domains.
    - **Multidirectional:** No single, normal path; healthy outcomes are achieved in many ways. Abilities can increase, decline, or remain constant.
    - **Plastic:** Within-person variability is possible for behaviors/development. Development can be modified by life circumstances, demonstrating the degree to which characteristics change or remain stable.
    - **Contextual:** Development varies across different environments (e.g., social, rural), influenced by biological make-up, physical environment, and social, historical, and cultural contexts.
    - **Multidisciplinary:** Understanding human development requires research from various disciplines (e.g., sociology, linguistics, neuroscience, medicine).
    - **Involves Growth, Maintenance, and Regulation:** Mastering life involves conflict and competition among these three goals.
    - **Embedded in History:** Influenced by historical conditions of the time period.
    - **Normative Age-Graded Influences:** Biological and environmental influences similar for individuals in a particular age group (e.g., childhood, puberty).
    - **Normative History-Graded Influences:** Biological and environmental influences common to people of a particular generation (e.g., Depression, AIDS epidemic).
    - **Non-normative Events:** Unusual occurrences with major impact on an individual, not applicable to most (e.g., death of a parent at a young age, serious illness, winning lottery).
- **1.6 Facts of Development:**
    - **Early Foundations are Critical:** Attitudes, habits, and behavior patterns established early influence later adjustment.

- **Role of Maturation and Learning:** Maturation (unfolding inherent traits) provides raw material for learning (development from experience/effort); development is influenced by their interaction.
- **Definite and Predictable Pattern:** Development follows orderly patterns (physical, motor, intellectual, speech) governed by laws:
  - **Cephalocaudal Law:** Development spreads from **head to foot**.
  - **Proximodistal Law:** Development spreads **outward from the central axis of the body to extremities**.
- **Individual Differences:** No two people react the same way to environmental stimuli, leading to individuality.
- **Characteristic Behavior in Each Phase:** Each phase has typical behaviors, marked by periods of equilibrium (easy adaptation) and disequilibrium (difficulty adapting).
- **Hazards in Each Phase:** Each stage has physical, psychological, and environmental hazards that involve adjustment problems; awareness helps prevention/alleviation.
- **Development is Aided by Stimulation:** Stimulation, especially when ability is normally developing, helps reach full potential.
- **Cultural Changes Affect Development:** Individuals are molded to conform to cultural standards/norms.
- **Every Stage Has Social Expectation:** Success in fulfilling expectations depends on adhering to family/societal rules.
- **Traditional Beliefs:** Affect judgments of others and self-evaluation, profoundly influencing development patterns.

- **1.7 Research Methods for Life Span Development:**

  - **Longitudinal Method:** Studies **developmental changes in the same group or individual over a period of time** (e.g., case study of children's classroom behavior).
  - **Cross-sectional Method:** Studies developmental changes by **testing individuals of different ages at the same time**; faster and cheaper, less subject dropout.
  - **Sequential Method:** Combines longitudinal and cross-sectional; tests people in a cross-sectional sample more than once to analyze differences over time for different groups; offers a more realistic assessment.
  - **Time Lag Method:** Studies **different age groups in different years to determine the effect of historical events on behavior**; rarely used due to time, subjects required.
- **1.8 Obstacles in Studying Life Span Development:**

  - **Representative Sample:** Difficulty in obtaining a diverse age range, especially newborns/infants (parental objection, mood) and older adolescents/adults (availability, shyness, privacy concerns).

- **Establishing Rapport with Subject:** Difficult to get full personal details; information accuracy is questionable, as subjects may provide false information.
- **Appropriate Methodology:** Wide age range and varied developmental areas require assorted methods. Cultural changes affecting physical/mental development patterns are a concern.
- **Accuracy of Data:** Data inaccuracy can present false information about normal development. Measures for intelligence, behavior, well-being, life satisfaction, or happiness may vary by age and method.
- **Ethical Aspects:** Respecting subject rights, obtaining **consent** from adults or parents/guardians for children is crucial.

Unit 2: Prenatal Development

- **2.0 Introduction**
  - **Definition:** The process of an embryo or fetus gestating during pregnancy, from **fertilization until birth**. Also referred to as fetal development.
  - **Factors Considered:** Genetically determined biological variables, non-genetically determined biological variables (e.g., lack of oxygen, malnutrition), child's past learning, immediate socio-psychological environment (parents, siblings, peers, teachers), and social/cultural milieu. Biological and environmental factors act in unison.
- **2.2 Beginnings of the Life**
  - **Conception:** Begins with the combination of male (sperm) and female (ovum/egg) sex cells.
  - **Chromosomes:** Sperm releases 23 chromosomes, ovum releases 23, creating a new individual with **46 chromosomes (23 pairs)**.
  - **Sex Determination:** The 23rd pair (sex chromosomes) determines sex. Females have **XX**, males have **XY**. The ovum always contains an X chromosome. A Y-bearing sperm results in a boy, an X-bearing sperm results in a girl. Sex is fixed at conception.
  - **Importance of Sex on Development:**
    - **Cultural Pressure:** Children face increasing pressure to adopt gender-appropriate attitudes and behaviors for social acceptance.
    - **Learning Experiences:** Influenced by sex (e.g., "sissy" vs. "tomboy" for playing gender-stereotyped games).
    - **Parental/Family Attitudes:** Traditional preference for boys can affect attitudes towards the developing infant.
  - **Preliminary Stages of Sex Cell Development:**
    - **Male Sex Cell:** Maturation and Fertilization.
    - **Female Sex Cell:** Maturation, Ovulation, and Fertilization.
    - **Maturation:** Chromosome reduction through cell division, forming haploid cells (23 chromosomes). Occurs after puberty.
    - **Ovulation:** Escape of one mature ovum during the menstrual cycle.

        - **Fertilization:** Union of male and female cells at conception, usually in the Fallopian tube.
- **2.3 Characteristics of Prenatal Period:**
    - **First and Most Important Period:** Foundation for later development.
    - **Shortest Period:** Approximately 270-280 days or nine calendar months (conception to birth).
    - **Heredity as Foundation:** Hereditary factors (physical and psychological traits) are established, though environmental conditions can foster or adversely affect their development.
    - **Sex Fixed at Conception:** Remains unchanged (except by surgery).
    - **Proportionally Greater Growth and Development:** More growth occurs prenatally than at any other time (e.g., single cell to 7-pound infant, weight increases 11 million times).
    - **More Hazardous:** High risk for environmental or psychological hazards affecting later development patterns.
    - **Attitudes Impact Development:** Parental attitudes, especially the mother's positive attitude, are crucial for normal development.
- **2.4 Periods of Prenatal Development:**
    - **Duration:** Approximately 10 lunar months (28 days each) or 9 calendar months, varying from 180-334 days. Average is 38 weeks (266 days).
    - **Three Stages:**
        - **2.4.1 Period of Zygote (Fertilization to end of second week):**
            - Begins when sperm fertilizes egg, forming a unique combination of genetic potential.
            - The **zygote** (fertilized egg) has **46 chromosomes** (23 from each parent).
            - Rapid **cell division (mitosis)** occurs, eventually forming a new human being.
            - This two-week stage is also known as the **germinal period**.
        - **2.4.2 Period of Embryo (End of second week to end of second lunar month / ~8th week):**
            - Develops into a **miniature human being**.
            - **Tissue differentiation** (cells taking on specific functions) occurs.
            - This is a **critical period**; the growing embryo is **most susceptible to damage from external sources (teratogens)** like viral infections (rubella), x-rays, radiation, and poor nutrition.
            - **Organ formation begins:** Heart (3 weeks), brain/spinal cord, gastrointestinal tract.
            - Teratogens can cause severe problems (e.g., limb absence, heart defects, facial clefts).
        - **2.4.3 Period of Fetus (End of second lunar month to birth):**
            - All organs and structures found in a full-term newborn are present.

        - **Weeks 9-12:** Head is nearly half the size, face well formed, eyelids close, tooth buds appear, **genitalia clearly male/female**.
        - **Weeks 13-16:** Second trimester begins, fine hair (lanugo), active movements (sucking, swallowing amniotic fluid), meconium forms, heart beats 120-150 bpm, brain waves detectable.
        - **Weeks 17-20:** Eyebrows/lashes appear, nails form. Mother feels movement ("quickening"), heartbeat detectable with stethoscope.
        - **Weeks 21-24:** Eye components developed, footprints/fingerprints form, body covered in vernix caseosa. Startle reflex, other reflexes (swallowing, coughing, sucking) appear.
        - **Weeks 25-28: Rapid brain development**, nervous system controls some body functions, eyelids open/close. Survival possible but high risk of complications.
        - **Weeks 29-32:** Development towards independent life (respiration movements), rapid increase in body fat, storage of iron/calcium/phosphorus. Bones fully developed but pliable.
        - **Weeks 33-36:** Body hair disappears. High chance of survival if born at 36 weeks.
        - **38 weeks:** Fetus is considered **full term**. Mother supplies antibodies for disease protection.
- **2.5 Environmental Influences (Teratogens):** Anything affecting the fetus's environment can influence development from conception.

    - **i) Mother's Diseases:** Can cause fetal death or defects. **German measles (rubella)** and cytomegalovirus are dangerous, potentially leading to stillbirth, deafness, prematurity, miscarriage, organ defects, and mental retardation. Other viruses like mumps, polio, influenza have teratogenic effects. Diabetic mothers have higher risk of malformations and respiratory difficulties. Blood poisoning can lead to premature or smaller babies.
    - **ii) Drugs and Chemicals:** Avoid unless medically recommended.
        - **Alcohol:** Greater consumption harms embryo/fetus; more than three drinks/day can cause permanent growth retardation, brain cell abnormalities, facial/joint/limb disfigurements, heart defects, mental retardation, attention deficits. Smaller daily consumption can impact attention/reaction time.
        - **Tobacco (Cigarette smoking):** High risk of premature delivery and low birth weight, leading to later growth and cognitive deficiencies.
        - Other drugs like antihistamines, aspirin (excess), barbiturates, heroin, lead, quinine, thalidomide, insulin (large doses) can cause deformities.
    - **iii) Radiation:** Larger doses are injurious to fetus, can cause spontaneous abortion. No completely safe level; natural radiation levels correlate with congenital abnormalities.
    - **iv) Abortion:** Termination of pregnancy (natural or medical). Can be performed by licensed physician before specified weeks. Modern techniques make early

abortion safer than childbirth. Opposition is moral/religious, not medical. (Note: The source mentions abortion as a "procedure to know the child gender" in a fill-in-the-blank, which is an oversimplification and potentially misleading, as gender determination is often for medical purposes, not solely for gender selection leading to abortion.)

- **v) Age of the Mother:** Higher risk for infant defects, prematurity, and death for mothers **over 35** (aged ova) or **under 18** (undeveloped reproductive system, poor prenatal care).
- **vi) Nutrition/Diet of the Mother:** Mother is the sole source of nutrition. Proper balance of proteins, fats, carbohydrates, minerals, vitamins is vital. Deficiencies linked to prematurity, low birth weight, stillbirth, growth retardation, poor mental functioning. Particularly harmful during first trimester, but also important in last trimester due to rapid brain growth.
- **vii) Stress in the Mother:** Can affect fetal development, though less than nutrition. Maternal emotions (via autonomic nervous system, endocrine glands, hormones) can pass through placenta and affect fetus. Anxiety in last month of pregnancy associated with motor depression in newborns.
- **viii) Rh Incompatibility:** Problem arises when Rh-positive male and Rh-negative female conceive an Rh-positive child. Mother's system may produce antibodies that destroy child's red blood cells, leading to erythroblastosis, death, or mental retardation.

Unit 3: Development During Infancy (Physical, Psychosocial, Cognitive and Linguistic)

- **3.2 Concept of Infancy Period**
  - **Definition:** The period from **birth to 2 years**. It is the "first part of life" or "early childhood".
  - **Characteristics:** Beginning and **fastest period of development**. Infants are helpless and dependent.
  - **Development Patterns:** Occurs in an orderly and predictable manner, following:
    - **Cephalocaudally:** Development takes place from **head to tail**.
    - **Proximodistally:** Development takes place from **trunk to extremities**.
  - **Key Psychologists:** Freud, Erikson, and Piaget highlighted different aspects of infant development.
- **3.3 Characteristics of Infancy Period:**
  - **Shortest Period:** From birth to two years, following the critical nine months in the womb.
  - **Crucial Adjustment Period:** Infants must adjust to outer surroundings quickly (usually within two weeks); difficult/premature births may require more time.
  - **Plateau in Development:** Growth and development from prenatal period halt temporarily after birth; infants may lose weight initially before gaining it again.
  - **Future Prediction:** Infancy is a period for predicting later development.

    - **Full of Hazards:** Both physical and psychological adjustments pose difficulties. Parental attitudes can crystallize during this time.
- **3.4 Adjustments During Infancy:**
    - **Temperature Changes:** Adjusting from 100°F in uterine sac to 60-70°F outside.
    - **Breathing:** Infant must begin to breathe independently after umbilical cord is cut.
    - **Sucking and Swallowing:** Must learn to get nourishment independently, as reflexes are imperfect at birth, potentially leading to initial weight loss.
    - **Elimination:** Organs begin to work soon after birth, taking over from umbilical cord elimination.
- **3.5 Hazards During Infancy Period:**
    - **1) Physical Hazards:**
        - **Complications at Birth:** Physical injury risk (e.g., anoxia from C-section leading to brain damage). Excessive medication during birth can also cause complications.
        - **Multiple Births:** Infants usually smaller and weaker due to crowding, often born prematurely.
        - **Post-maturity:** Large fetus size at birth may necessitate instruments/surgery, posing hazards.
        - **Pre-maturity:** High risk of infant death and brain damage; respiratory issues (anoxia) can have long-lasting effects.
    - **2) Psychological Hazards:**
        - **Traditional Beliefs about Birth:** Affect child's development (e.g., difficult birth = difficult life).
        - **Helplessness:** Psychological hazard, especially for firstborns.
        - **Attitude of Parents:** Can change due to gender preferences, crying, nourishment difficulties, birth complications, or unexpected multiple births. Mother's attitude is particularly important due to direct contact.
- **3.6 Physical Development in Infancy:**
    - **Physical Growth:** Rapid in the first year (weight doubles in 6 months, triples in a year). Significant expansion of head and chest. Bones harden, fontanelles calcify (small one ~3 months, large one up to 18 months). Brain reaches 75% of adult weight by end of second year.
    - **Factors Affecting Growth:** Environmental conditions (e.g., severe nutritional deficiency leading to irreversible impairment; overfeeding leading to obesity) and genetic endowment. Human milk provides basic nutrition.
    - **Sleep Patterns:** Newborns sleep almost constantly; waking periods increase, establishing regular schedules by three months.
    - **Maturation:** Universal sequence of biological events in CNS permitting psychological function, assuming health and environment. Sets limits on earliest appearance (e.g., puberty onset influenced by nutrition).

- **3.7 Psychosocial Development in Infancy (Erik Erikson):**
    - **Erikson's Theory:** Describes the impact of social experience across the lifespan.
    - **Ego Identity:** Conscious sense of self developed through social interaction, constantly changing with new experiences.
    - **Competence:** Motivation for behaviors/actions. Mastery leads to ego strength, poor management leads to inadequacy.
    - **Conflict:** Each stage has a conflict, serving as a turning point (high potential for growth or failure).
    - **Stage 1: Trust vs. Mistrust (Birth to 1 year):**
        - **Most fundamental stage.** Infant is purely dependent on caregivers.
        - **Successful Trust:** Achieved when basic needs are met with warmth, regularity, dependable affection, leading to feelings of safety and security. Foundation for lifelong expectation of a good world.
        - **Mistrust:** Results from inconsistent, emotionally unavailable, or rejecting caregivers, leading to fear and belief that the world is unpredictable.
        - **Virtue:** Proper balance leads to **hope** (belief that things will work out).
    - **Other Social Behaviors:**
        - **Attachment:** Infant seeks love/attention from mother. Early attachment is "indiscriminate" (seeking stimulation). By six weeks, infants recognize caretakers. **"Emotionally toned relationship or tie to the mother"** (Ainsworth et al., 1978) that provides basis for healthy emotional/social development.
        - **Smiling:** Early communication. First a facial exercise, then indiscriminately bestowed. Mother's responsive smile transforms it into a social interaction (first real social interaction). Social smile appears at 7-8 weeks; by 3 months, smiles at almost any face, inviting adult interaction and contributing to bonding.
        - **Anxiety:** Develops with attachment. Infant becomes aware of mother as special, then fears losing her. Crying/searching indicate attachment (e.g., crawling after mother, crying when she disappears).
        - **Fear of Strangers (Stranger Anxiety):** Direct result of first attachment. Upset by unfamiliar adults, especially without mother present. Stares, cries, shows distress. Disappears by end of first year as child contacts more relatives.
- **3.8 Cognitive Development in Infancy (Jean Piaget):**
    - **Cognition Definition:** Mental activities for acquisition, processing, organization, and use of knowledge (detecting, interpreting, classifying, remembering, evaluating, inferring, imagining).
    - **Infant Competence:** Remarkably competent from birth (senses, pain, touch, bodily position).

- **Piaget's Theory:** Most influential. Knowledge helps adaptation to environment. Child is active, curious, inventive. Cognitive progress from maturing child's interaction with the world.
- **Stages of Cognitive Development:** Piaget identified four:
  - **i) Sensory Motor Stage (Birth - 2 years):** Infant uses senses and motor abilities (reflexes to complex skills) to understand the world. Divided into six sub-stages:
    - **Reflexes (birth-1 month):** Understanding through inborn reflexes (sucking, looking).
    - **Primary Circular Reactions (1-4 months):** Repetition of own action that serves as a stimulus to which it responds (e.g., thumb sucking).
    - **Secondary Circular Reactions (4-8 months):** Intentional repetition of an action to trigger a response in the environment (e.g., shaking a rattle to hear noise).
    - **Coordination of Secondary Reactions (8-12 months):** Focus on desired object; responses become more coordinated and complex (e.g., moving an obstacle to get a toy).
    - **Tertiary Circular Reactions (12-24 months):** Trial-and-error experimentation (e.g., trying different ways to make a toy work).
    - **Early Representational Thought:** Development of symbols to represent events or objects (e.g., pretending to talk on a phone).

- **3.9 Linguistic Development in Infancy:**

  - **Process:** Starts early, simple to complex. Babies read lips and discriminate speech sounds by four months. Start without language, then babbling. Speech replaces gestures/babbling for communication.
  - **Self-Awareness:** Use of names and personal pronouns ("I," "me," "my") represents self-awareness.
  - **Vocabulary Growth:** Rapid after first word (around end of first year). Babbling ("baba," "dada") at 4-8 months. First understandable words ("mommy," "dog") around 12 months. Two-word combinations ("mommy milk") by 18 months.
  - **Four Main Components of Language Development:**
    - **i) Phonology:** Rules for speech sounds structure/sequence.
      - **Cooing (2 months):** Mostly vowel sounds.
      - **Babbling (4 months):** Repetitive consonant-vowel combinations.
      - Babies understand more than they say. Recognize correct pronunciation of familiar words by 1-2 years and simplify pronunciation.
    - **ii) Semantics:** Vocabulary and how concepts are expressed.
      - **Comprehension precedes production** (5-month lag).
      - **Vocabulary spurt (18-24 months):** Fast mapping (learn many new words quickly). Majority are object (nouns) and action (verbs) words.

        - **By 3-5 years:** Difficulty using words correctly (under-extensions, over-extensions), coin words. Understand metaphors.
    - **iii) Grammar:** Two parts:
        - **Syntax:** Rules for arranging words into sentences.
        - **Morphology:** Use of grammatical markers (tense, voice).
        - **1-2 years:** Telegraphic speech (two-word combinations, e.g., "mommy milk").
    - **iv) Pragmatics:** Rules for appropriate and effective communication.
        - **Skills:** Using language for greeting/demanding, changing language for different listeners, following rules (turn-taking, staying on topic).
        - **Birth-1 year:** Joint attention (sharing attention), turn-taking.
        - **1-2 years:** Conversational turn-taking, topic maintenance.

Unit 4: Early Childhood (Physical, Psychosocial, Cognitive and Linguistic)

- **4.2 Meaning of Early Childhood:**
    - **Definition:** Period from **2-6 years**. Also referred to as **toddlerhood** and sometimes **play age**.
    - **Emphasis:** Children learning through play. Laying foundation for subsequent learning (brain development).
    - **Terminology:** "Preschool" or "kindergarten age" for 3-6 years, emphasizing education. Parents are integral to the education process.
- **4.3 Characteristics of Early Childhood:**
    - **Behavioral Problems:** Parents may find behavioral problems (obstinacy, stubbornness, disobedience, negativistic, antagonistic) more troublesome than infant physical care.
    - **Toy Age:** Children are often engaged with toys, which aid education.
    - **Independence:** Physically and mentally independent; school-going age.
    - **Self-Sufficiency:** Children become more self-sufficient, independent, and develop self-esteem.
    - **Social Behavior Foundations:** Develop more organized social life for first grade adjustment.
    - **Holistic Development:** Develop physical, cognitive, emotional, and social aspects.
- **4.4 Hazards During Early Childhood:**
    - **1) Physical Hazards:**

        - **Illness:** Highly susceptible to respiratory illnesses and infectious diseases. Extended sickness can impede skill learning.
        - **Accidents:** High risk of deaths. Boys are more prone. Common injuries: cuts, burns, infections, broken bones. Can cause temporary or permanent disability.
        - **Obesity:** A health hazard. Children with endomorphic body builds or those fond of junk food are prone, increasing risk of diabetes and heart attacks. Overweight can lead to reduced active play and missed skill acquisition.
    - **2) Psychological Hazards:**
        - **Speech Hazards:** Unclear communication can lead to feelings of inadequacy and inferiority.
        - **Social Hazards:** Communication problems can lead to unpopularity among peers, loneliness, and lack of opportunities to learn peer-approved behaviors. Experiences of discrimination can lead to biased behaviors and minimized social contact.
        - **Play Hazards:** Isolation from playmates (geographical or forced solitary play) can lead to rejection by other children, underdeveloped motor skills, and feelings of handicap/inferiority.
        - **Moral Hazards:** Inconsistent discipline slows conformity to social expectations, causing confusion when different people have different views on behavior.
- **4.5 Growth and Development in Early Childhood:**
    - **Complementary Processes:** Growth (qualitative bodily changes like height/weight) and Development (qualitative and quantitative changes like intelligence, creativity, language, and adaptation).
    - **Principles of Developmental Change:**
        - Orderly sequence.
        - Each child passes through stages with essential characteristics.
        - Individual differences in rate and pattern.
        - Human develops as a unified whole, but each body part develops at a different rate.
        - Result of **interaction between maturation and learning**.
- **4.6 Physical Development in Early Childhood:**
    - **Overall:** Rapid changes in physical, cognitive, and language development.
    - **Height:** Growth rate slows; average 2.5 inches per year.
    - **Weight:** Average 3-5 pounds annual increase. By age 6, six times birth weight. Body fat declines.
    - **Body Build:** Differences apparent: endomorphic (flabby/fat), mesomorphic (sturdy/muscular), ectomorphic (thin). Boys gain more muscle, girls more fat.
    - **Motor Skills:** Gross motor skills (running, skipping, jumping) and fine motor skills (turning pages, writing, drawing) progress rapidly.

    - **Brain Development:** Most important physical development; nervous system grows.
    - **Body Proportion and Shape:** Average preschooler needs 1700 calories/day. Diet affects skeletal growth, body shape, disease susceptibility.
    - **Teeth:** Last four baby teeth erupt in first 4-6 months. Permanent teeth begin to replace baby teeth in latter half of early childhood.
- **4.7 Psychosocial Development in Early Childhood (Erik Erikson):**
    - **Erikson's Psychosocial Theory:** Describes social experience impact on development. Children develop self-conscious emotions.
    - **Stage: Autonomy versus Shame and Doubt:** Children begin to evaluate themselves.
        - **Autonomous/Positive:** See self as good, valuable, capable.
        - **Negative/Ashamed:** Feel worthless, incapable.
    - **Social Skills:** Increase with self-awareness, communication, and understanding others' thoughts/feelings. Children learn to modify/express emotions to fit social situations (e.g., not having tantrums at school, acting pleasant despite shyness).
    - **Outcome:** Successful completion leads to security and confidence; failure results in inadequacy and self-doubt.
    - **Parental Role:** Parents act as consultants, providing reasonable/consistent guidelines, which make children feel secure and loved. Mothers often preferred companions, while fathers encourage independence and assertiveness.
- **4.8 Cognitive Development in Early Childhood (Jean Piaget):**
    - **Piaget's Processes:**
        - **Assimilation:** Using or transforming the environment to fit into **preexisting cognitive structures** (e.g., sucking on a larger bottle with a small-bottle schema).
        - **Accommodation: Changing cognitive structures** to accept something new from the environment (e.g., modifying sucking schema for a pacifier to fit a bottle).
        - Both used simultaneously and alternately. Schemas become complex **structures**.
    - **Stage: Pre-operational Stage (2-7 years):** (Also called Play age and Early Childhood)
        - **Intelligence:** Demonstrated through **use of symbols**, maturing language, developed memory and imagination.
        - **Thinking:** Non-logical, non-reversible.
        - **Egocentric Thinking:** Predominates. Children have difficulty accepting others' viewpoints.
        - **Ages 2-4:** Develop **symbolic reasoning** (ability to picture absent objects). Egocentrism starts strong but weakens. Magical beliefs are constructed.

        - **Ages 4-7:** Develop **intuitive thought** (primitive reasoning, "why" questions). Starting school is a major landmark.
    - **Educational Implications:** Piaget's theory provides foundation for constructive learning (discovery learning, supporting child's interests). Teachers/parents should challenge abilities and use concrete experiences (manipulatives, group work, field trips).
- **4.9 Linguistic Development in Early Childhood:**

    - **Importance:** Language is a powerful tool for cognitive development, communication, and problem-solving. Unclear communication can create hazards.
    - **Vocabulary Growth:**
        - By **3 years:** 300-1,000 spoken words.
        - By **5 years:** ~1,500 words. Can produce 5-7 word sentences, use past tense, tell familiar stories with picture cues.
    - **Consonant Mastery:**
        - By **6 years:** f, v, sh, zh, th, l. Speech should be completely intelligible and socially useful. Can tell well-connected stories from pictures.
        - By **7 years:** s-z, r, voiceless th, ch, wh, soft g. Handle opposite analogies easily (girl-boy). Understand terms like alike, different, beginning, end. Can tell time to quarter hour, simple reading/writing.
    - **Cognitive Concepts:** By end of age seven, basic understanding of less concrete concepts like time and money. Eight-year-olds still reason concretely, difficulty with abstract ideas.

Block 2: Physical, Cognitive, Social, Emotional and Moral Development in Middle Childhood

Unit 1: Physical Development (6-11 Years)

- **1.0 Introduction:**

    - **Terminology:** Also called **"middle childhood," "school years," or "elementary school age"**.
    - **Characteristics:** Period of **slow but steady physical growth**, perfection of motor abilities, rapid cognitive and social skills development.
    - **Individual Differences:** Each child has a unique growth timetable.
    - **Social Aspects:** "Gang-age" – major concern is acceptance by peers. Child faces new social expectations and must master developmental tasks (reading, writing, calculating, social attitudes). Failure to master these tasks leads to poor personal/social adjustment later.

- **1.2 Physical Growth in Early School Years:**
    - **Growth Rate:** Relatively **slower and uniform** compared to earlier periods, until pubertal changes begin.
    - **Body Size (Height and Weight):**
        - Height: 5-6 cm annually.
        - Weight: Slow and fairly uniform.
        - Ages 6-12: Average growth of 2-2.5 inches and 3-6 pounds annually.
        - **Girls are slightly heavier than boys** and begin their pubertal growth spurt sooner (around age 9).
        - **Initial growth spurt indication:** Sudden increase in foot length/shoe size.
    - **Factors Affecting Body Size:**
        - **Family Influences:** Hereditary and environmental factors.
        - **Nutrition:** Well-nourished children are taller and reach puberty sooner. Poor nutrition can prevent hereditary growth potential.
        - **Emotional Disturbances:** Can inhibit growth hormone production, delaying growth.
        - **Socio-economic Status:** Children from lower SES homes are smaller.
        - **Health:** Good health and fewer illnesses lead to taller children.
        - **Endocrine Functioning:** Normal functioning results in normal size; deficiency can lead to dwarfism, excess to giantism.
        - **Sex:** Girls tend to be slightly heavier and mature sooner.
    - **Brain Development:**
        - Brain is **90% of adult weight by 6 years**.
        - **Temporal and parietal lobes** (language, spatial relations) develop fastest between 6-puberty.
        - **Right hemisphere growth spurt (8-10 years):** Improves sensation, perception, spatial orientation, time perception, music appreciation, recognition of objects/faces, non-verbal communication, emotions (empathy, humor), vigilance, attention, left-side body movements, planning, organizing.
- **1.2.3 Motor Skills in Growth and Development (6-11 Years):**
    - **Overall:** Fairly **smooth and strong motor skills**, though coordination (eye-hand), endurance, balance, and physical tolerance vary widely. Genetic background, nutrition, exercise influence development.
    - **Secondary Sexual Characteristics:** Begin to develop (girls: breast development 8-10 years, hips, menarche 11-12 years; boys: testicle enlargement 9-11 years, later physical maturity). Emotional challenges arise.
    - **Body Proportions:** Head becomes smaller in proportion (from 1/4th at newborn to 1/6th at 6-8 years).
    - **Muscle Development:** Gross muscles (arms, legs) and fine muscles (fingers, toes) develop rapidly.

- **1.2.4 Development of Teeth, Bones and Muscles:**
  - **Teeth:** By middle childhood, children have **28 permanent teeth** (milk teeth replaced).
  - **Bones:** All bones formed by middle childhood, growing in size and strength. Sufficient calcium makes them strong, supporting high activity levels.
  - **Muscles and Fat:** Girls have more fat than muscles, while boys have more muscles (explaining greater strength).

- **1.3 Motor Development:**

  - **Muscular Coordination:** Learned movement due to muscular control.
    - **Gross Muscular Coordination:** Movement of large muscles (running, balancing, skipping, climbing).
    - **Fine Muscular Coordination:** Movement of small muscles (picking pencil with index finger/thumb, writing, needlework, painting).
  - **Developmental Sequence:** Gross motor coordination is learned before fine motor coordination.
  - **Sensitive Period:** Time when a child can learn a specific activity/skill most effectively. Forcing activities before muscles are ready (especially fine muscles) can cause damage (e.g., writing before 4.5-5.5 years).
  - **Activities & Skills (6-11 Years):** Throwing/catching balls, coordinated running, balancing on one foot, jumping (high, two legs), skipping, hopping, jumping hurdles.
  - **1.3.2 Disorders in Physical Development (6-11 Years):**
    - **Attention Deficit Disorder (ADD) / Hyperactive or Hypoactive:**
      - **Hyperactive:** Distractible, impulsive, irritable, moody, slow learning, inattentive, constant movement, chaotic behavior, forgetful, unable to do sequential tasks, unpopular.
      - **Hypoactive:** Less than normal activity, excessive daydreaming, quiet, inattentive to tasks, may go unnoticed.
    - **Execution of Motor Skills:** Wide individual differences in complex motor tasks. Improvements in gross motor skills: speed, power, coordination, agility, balance. Can ride scooters, bikes, skates. Learn new skills (run faster, hop/jump, throw/catch/kick accurately). Team sports (basketball, football, dancing, swimming) refine skills.
    - **Disorders:** Congenital/acquired deficits, injuries leading to deformity, high fever/illnesses impacting peer equivalence, polio, brain damage leading to poor coordination.
  - **1.3.3 Improvement in Control and Coordination of Fine Motor Skills:**
    - Perform progressively neater and smaller tasks (musical instruments, sewing, knitting, detailed drawing) requiring dexterity and fine motor control.
    - **Motor skills depend on:** Body size, strength, brain maturation, temperament (energy, venturesomeness, aggressiveness, persistence),

attitude toward body, eagerness for group functions. Shy children or those with low self-esteem may miss opportunities to develop skills through peer group interaction.

- **Participation:** Children actively participate in school and neighborhood activities (scouts, teams), fostering friendships and shared interests.
- **Progress Summary:** Steady and sustained growth. Increased motor skill execution. Individual variation. **Secular trend** (children today taller and mature earlier than previous generations). Self-attitude related to body size/shape; others' reactions become important.

- **1.4 Body Proportions:**

  - **Changes:** Legs grow faster. Disproportion of large head decreases; lower face increases size. Trunk elongates and slims. Chest broadens/flattens, neck longer, shoulders drop, pelvis increases. Arms/legs thin. Hands/feet grow slowly, often longer for boys.
  - **Weight Loss:** Children lose "baby fat"; faces become slimmer/narrower.
  - **Teeth:** "Toothless smiles" common (baby teeth lost, replaced by permanent teeth by 11-12 years). All permanent teeth except second/third molars.
  - **Eyes:** Not final shape/size at 6. Slightly farsighted at 6-8, corrects by 8-10. Binocular vision established by 6. Reading best delayed until ~6 years.
  - **Posture:** Marked improvements (erect bearing replaces rounded shoulders, spinal curvature, prominent abdomen), leading to efficient arm/leg use.
  - **Brain Growth:** Essentially complete by 10 or 12.
  - **Bones:** Flexible, more space at joints, but ossification incomplete (less resistant to breaks/fractures/muscle pulls than adults).
  - **1.4.1 Muscle and Fat:** Muscles increase in size/strength, but still functionally immature compared to adolescents. Sports, dance, skating aid development.
    - **Heart:** Grows more slowly during school years, proportionally smaller.
    - **Body Fat:** Accounts for 15% of body weight. Girls retain more fat. Appetite increases after 6, risk of obesity; overweight children miss active play/skill acquisition.
    - **Gastrointestinal System:** Mature. Fewer stomach upsets, steadier blood sugar, greater capacity than preschoolers. Caregivers should minimize junk food.
    - **Lungs:** Grow until ~8 years, capacity increases, respiratory rate decreases.
    - **Bowel and Bladder Control:** Usually well established.
    - **Ear and Hearing:** Well developed, auditory sensitivity improves.
- **1.5 Motor Skills Development:**

  - **Ideal Age:** Childhood is ideal for motor skill learning due to pliability, fewer conflicting learned skills, adventurousness, enjoyment of repetition, and more free time.

- **Refinement:** Smooth coordination of small/large muscles refined (reading, writing, team games). Children perfect six basic behaviors: running, jumping, sequencing foot movements, balancing, throwing, catching.
- **Jumping:** Good index of coordination/strength. Boys often exceed girls in vertical jump height; girls superior in jumping/hopping into grids. Rapid improvements 6-9 years.
- **Controlled Activities:** More energy channeled into sports, cooperative play. Children become more competitive, form larger/complex groups. Benefits of team sports: social contact, self-improvement, teamwork, physical fitness.
- **Handedness:** Well established by 6.
- **Artistic Skills:** Small-muscle ability and artistic skill improve steadily until 12 (musical instruments, handicrafts like ceramics, needlework, painting, model building). Girls have greater hand/finger dexterity.
- **Eye-hand Coordination:** Substantially improves. By 8, better movement planning; by 10, increased control over grasp speed/direction.
- **Clumsiness:** School-age children may become clumsier and less coordinated during growth spurts due to maneuvering larger bodies and preoccupations.
- **1.5.1 Major Motor Developments Summary:** Improved running, jumping, foot sequencing, balancing, throwing, catching. Addition of skilled movements (sports, games, music, hobbies). Improved reaction time, eye-hand coordination. Common competitive sports. Improved left-right discrimination. Well-established handedness. Improved fine motor skills (girls greater dexterity).

Unit 2: Cognitive, Social, Emotional and Moral Development (6-11 Years)

- **2.0 Introduction:**
    - Referred to as the "latency period," but years 6-12 are "action packed" with motion and emotion as children confront diverse demands.
- **2.2 Cognitive Development:**
    - **"5 to 7 Shift":** Dramatic change in thought processes; pre-operational thought replaced by rudimentary logic. Transition from induction to logic.
    - **Brain Development:** Impressive changes set the stage. **Cross-model zones** (nerve networks for information flow between brain parts) are well developed by 5-6 years, linking sensory events (e.g., associating orange with color, smell, flavor, spelling).
    - **Reciprocal Relationship:** Brain maturation supports cognitive changes, and cognitive activity can accelerate brain activity.
- **2.2.1 Piaget's Theory of Cognitive Development: Concrete Operation:**
    - **Third Stage:** The **ability to think logically about concrete or real-world events and experiences** is the hallmark.

    - **Operations:** Flexible mental actions that combine to solve problems. These are powerful rules transforming information (e.g., mathematical operations, relations among categories).
    - **Concrete Nature:** Operations are limited to concrete objects, abstract concepts are difficult.
- **2.2.2 Decentration:**
    - A significant operation mastered in school age. Children can **decenter their thinking**, taking into account several aspects or events simultaneously.
    - **Hierarchical Organization:** Concrete operational children organize the world into hierarchies, understanding that a thing can belong to more than one dimension at once (e.g., "more children or more people?" – 9-year-olds correctly identify "more people" as both children and adults are people, while 4-year-olds focus only on one dimension).
- **2.2.3 Conservation Tasks:**
    - **Definition:** Tasks testing a child's knowledge that an entity's identity of matter remains the same even if its physical characteristics change.
    - **Ability:** Concrete operational children are no longer fooled by perceptual appearance (e.g., flattened clay still has the same weight).
    - **Developmental Sequence:**
        - **Numbers:** Around **6 or 7 years**.
        - **Mass and Length:** Around **7 or 8 years**.
        - **Weight:** Around **9 or 10 years**.
        - **Volume:** Around **14 or 15 years**.
    - **Horizontal Decálàge:** Piaget's term for the developmental lag in conservation abilities, meaning sequential mastery of concepts within a single stage.
    - **Concepts for Conservation:**
        - **Identity:** If form changes but nothing is added/taken, amount stays same.
        - **Reversibility:** Something changed can return to original state by reversing process.
        - **Reciprocity:** If mass is constant, change in one dimension necessitates change in another (e.g., flattened object becomes wider as it thins).
    - **Benefit:** Provides new sense of stability, security, and confidence, relying on knowledge rather than perception.
- **2.2.4 A New Egocentrism:**
    - Replaces preschool egocentrism. School age children realize their way of thinking isn't the only way and can appreciate others' viewpoints.
    - **"Three Mountain Task":** Young school-age children can occasionally select the correct drawing of what a doll might see, with accuracy improving by 11-12 years.
    - **Cognitive Bias:** Once a hypothesis is formed, they tend to force contradictory facts into it rather than changing the hypothesis.

- **2.2.5 Logical Reasoning:**
    - **Inductive Reasoning:** Develops in school age; reasoning from specific observation to a general principle. Applied in learning rules/operations, and forms foundation of empathy.
    - **"If-then" Conditions:** Great advances in understanding between 3rd and 5th grade.
    - **Deductive Logic:** Does not appear until formal operations stage (12 to adult).
- **2.3 Concept Formation:**
    - **Number:** Understanding of one-to-one correspondence complete by 6 or 7 years (e.g., six is always six regardless of representation).
    - **Time:** Difficulty before 8; more precise understanding and ability to classify past/future events after 8.
    - **Spatial Operations:** Difficulty understanding distance (basic units of measurement) and navigating unfamiliar spaces before school age. Older children can draw maps after thorough exploration, but struggle with cognitive maps for directions/object location even at 10.
    - **Classifications:** Class inclusion (addition of classes) well-developed. Can form class hierarchies and understand multiple identities. Most children can understand multiplication of classes by 6 or 7.
    - **Seriation:** Capable of sequencing and ordering objects by measurable dimensions (e.g., weight, size).
- **2.3.1 Information Processing Approach to Cognitive Development:**
    - Suggests concrete logic results from improved attention, perception, memory, and problem-solving skills.
    - **Attention:** Improves during school years, focusing on relevant information and ignoring distractions. Interest enhances attention.
    - **Perception:** Concrete operational logic influences sensory information organization/interpretation. Children can't spontaneously alternate between figure/ground until 10-11 years, requiring reversibility. Older children search visual environment thoroughly/systematically.
    - **Memory Capacity and Storage:** Hold more information, better at mentally organizing material. Rehearsal occurs spontaneously. Memory improved by organizing/categorizing. Older children use elaboration (improving memory by changing form/associating with other info/images). **Metamemory** (awareness of memory) develops.
- **2.3.2 Language Development:**
    - **Vocabulary:** Continues to expand; improved understanding of words and meanings. Training in word relationships/structures develops extensive vocabularies.

- **Language Structure:** Refine understanding of sentence organization. 6-7 year olds confused by irrelevant information, complex constructions, implied meanings.
- **Behavioral Link:** Children with language difficulties more likely to exhibit aggressive behavior.
- **Communication Effectiveness:** Develops with cognitive growth; asking for clarification, persuading, sensitivity to listener's needs, enhanced vocabulary.

- **2.4 Social Development:**

  - **Expanding World:** From families to schools, siblings to peers/friends. Adults (teachers) become more important.
  - **2.4.1 Relations with Parents and Siblings:**
    - **Independence:** Children want more time with friends, less with family. Desire for decision-making.
    - **Parental Role:** Parents challenged to provide guidance without being restrictive. Parental pride/faith fosters resilience; belittling leads to failures/low self-esteem. Parents become "consultants".
    - **Responsibility:** School-age children ready for more responsibility (chores, personal items), developing skills, self-confidence, appreciation for daily living tasks. Children with responsibilities are more nurturing/mature.
    - **Value Development:** Affected by parental behavior. Children test values against peers; may side with peers when values conflict. Values more resistant to change when reasons are known.
    - **Self-esteem:** Most important predictor of happiness/effective functioning. Influenced by self-perception, home/school experiences. Adults can help by making children feel powerful, competent, virtuous, significant. Consistent guidelines provide security.
    - **Parental Preferences:** Mothers preferred companions. Fathers encourage independence/assertiveness; mothers train interpersonal skills.
    - **Sibling Relationships:** Significant during middle childhood. Teach/help each other, practice social skills (gratitude, annoyance, surprise, fear). Younger siblings emulate older ones.
  - **2.4.2 Peer Group:**
    - **Definition:** Group of equals, same-age, same-sex children.
    - **Structure:** Small, loosely organized (6-9 years); more structured, formalized, exclusive, cohesive (5th/6th grade).
    - **Expectations:** Follow social norms (courtesy, fair play, respect).
    - **Conflict Resolution:** Girls favor cooperation, boys favor competition.
    - **Agents of Socialization:** Transmit attitudes/values, influence behavior through modeling/reinforcement.
    - **Conformity:** Mainstay; children conform due to valuing peers' opinions more than adults'.

        - **Social Competence:** Ability to sense social situations, responsiveness, understanding relationship development.
        - **Socio-metric Status (Peer Acceptance/Rejection):**
            - **Socio-metric stars:** Liked by all.
            - **Mixers:** Interact often, some liked, some not.
            - **Teachers negatives:** Conflict with teachers, some liked, others not.
            - **Tuned out:** Not involved, ignored.
            - **Socio-metric rejections:** Disliked and actively rejected (withdrawn or aggressive).
        - **Friendship Development (Selman):**
            - **Playmates (3-7):** Those who play together.
            - **Assistants (4-9):** Those who help each other.
            - **Cooperators (6-12):** Cooperate, share goals, make compromises.
            - **Intimates and Mutual Supporters (9-15):** Share goals/values, provide intimacy/support; strong friendships survive disagreements.
            - **Dependent but Autonomous (12+):** Adult-like understanding of mutual dependence while maintaining individuality.
    - **2.4.3 Social Cognition:**
        - **Definition:** Knowledge of others' emotions, motives, intentions.
        - **Selman's Stages of Perspective-Taking:**
            - **Egocentric (3-7 years):** No other perspective, others feel as I do.
            - **Social informational (4-9):** Others have a view, but feel as I do; aware but don't understand.
            - **Self-reflective (6-12):** Begin to infer other views; understand different viewpoints.
            - **Mutual (10-12):** Can switch perspectives.
            - **Social and conventional (12-adulthood):** Analyze perspectives in abstract terms.
    - **2.4.4 Self-esteem:** Self-appraisal, reflecting discrepancy between actual and ideal competence. Assessed in scholastic, athletic, physical appearance, social acceptance, and morality. High self-worth associated with happiness, low with sadness/depression.
- **2.5 Moral Development:**

    - **2.5.1 Piaget's Ideas about Moral Development:**
        - **Heteronomous Morality / Moral Realism (6-7 years, with concrete operations):** Rules are **unchangeable, absolute, imposed by external authority**.
            - **Egocentric Beliefs:**
                - **Imminent Justice:** Wrongdoing invariably leads to punishment.

            - **Objective Consequences:** Morality judged by objective consequences, not intentions.
            - **Absolutism:** Only one correct moral conclusion per circumstance.
        - **Autonomous Morality / Morality of Cooperation (~10 years):** Less egocentric, realize rules are **not fixed but arbitrary and can change**. Personal decisions about obeying rules.
            - Moral authority of adults replaced by **cooperation and mutual understanding**. Motives, rules, specific situations considered in judgment.
            - Praise/punishment should be non-arbitrary.
    - **2.5.2 Kohlberg's Theory of Moral Development:**
        - Refines Piaget's ideas using moral dilemmas (e.g., **Heinz dilemma**). Focuses on *reasons* for answers.
        - **Three General Levels (two stages each):**
            - **(i) Preconventional Level (Preschool/Early Childhood focus):** Moral reasoning influenced by obedience and punishment, and satisfying personal needs.
                - Stage-1: Punishment-obedience orientation.
                - Stage-2: Instrumental-exchange orientation.
            - **(ii) Conventional Level (School Years focus):** Shift from egocentric thinking; concern with **appearing "good"** and **law and order**. Rule-breaking inherently immoral as it creates chaos.
                - Stage-3: Good-boy-nice-girl orientation.
                - Stage-4: System-maintaining orientation.
            - **(iii) Postconventional Level (Later Development):**
                - Stage-5: Social-contract situation.
                - Stage-6: Universal-ethical-principles orientation.
    - **2.5.3 Moral Judgments and Moral Behavior:**
        - Supporting rigid moral standards doesn't guarantee ethical behavior.
        - 9-11 year olds quick to find excuses for rule infractions.
        - Moral decision-making involves coordinating conflicting needs/motives (culture's laws, peer morality, parental/teacher guidelines, self-interest).
        - Benefits from practice, maturity, and specific instruction on generalizing principles.
- **2.6 Emotional Development:**

    - **Changes with Maturity:** Emotional expressions change. Patterns vary and are affected by health, intellect, environment, social reactions.
    - **Parenting Styles:** Authoritarian training encourages anxiety/fear; permissive/democratic training encourages curiosity/affection.
    - **Socio-economic Status:** Low SES children tend to have more fears/anxiety.
    - **Gender Differences:** Girls may cry/have temper outbursts; boys may be sullen/moody.

    - **School Adjustment:** Emotionality subsides with school adjustment due to defined roles, outlets for energy (games/sports), and reduced frustration from improved skills.
- **2.6.1 Common Emotional Patterns:**
    - **Fear:** Gradual shift from typical to general fears (fanciful, supernatural, dark, death, injury, elements, fictional characters, failure, ridicule, being different). Overt responses curbed by social pressure; expressed indirectly (motor discharge, withdrawal, imaginary ills, shyness, embarrassment, worry, anxiety).
    - **Anxiety:** Develops later than fear, increases during 4th-6th grade. Expressed as depression, nervousness, irritability, mood swings, restless sleep, quick anger, increased sensitivity to others. Anxious children feel insecure.
    - **Anger:** More frequently expressed than fear. Aroused by thwarted desires, interrupted activities, faultfinding, teasing, unfavorable comparisons.
        - **Impulsive responses:** Aggressive behavior, temper tantrums (decrease with age).
        - **Inhibited responses:** Withdrawn, acting hurt, sullen, self-pitying, threatening to run away.
    - **Curiosity:** Instinctive foundation of intellectual life. Children ask "what, why, how" about everything. "Questioning age" replaced by reading if it answers questions.
    - **Joy, Pleasure, Delight:** Pleasant emotions. Expressions range from quiet contentment to exuberance. Children learn to express joy socially appropriately (e.g., no gloating). Success (academic, sports) is pleasant. Company of friends/play are delightful.
    - **Emotional Tolerance:** Ability to accept/adjust to unpleasant emotional experiences; essential for emotional balance.

Unit 3: Schooling and Development

- **3.0 Introduction:**
    - All aspects of development (physical, cognitive, social, emotional, moral) grow simultaneously.
    - **Schooling:** Formal institution that contributes to intellectual, emotional, social, and physical transformation of children into responsible citizens. It systematically nurtures curiosity and teaches basics, fostering a desire to learn more.
    - **Focus:** Development of **secondary abilities** (e.g., writing, playing violin) through explicit instruction and practice, differentiating them from primary abilities (mother tongue, sense perception).
- **3.2 School and the Life of Children:**
    - **Aims of Education (Bertrand Russell):**
        - **Cultivation of Intelligence:** Including actual knowledge and receptivity to knowledge.
        - **Discovery of Special Aptitudes:** To be developed in later years.

        - **Learning Basic Subjects:** What children ought to know and the order (easiest first).
        - **Avoid Severe Mental Effort:** Not before age seven.
    - **Education Centered at Child's Life (John Dewey):**
        - Education should adopt children's spontaneous ways of learning from surroundings. Opposed "drawing out" something hidden from child's mind.
        - Role of education is **giving direction to children's activities**.
        - **Instincts to Recognize and Shape:**
            - **Social Instinct:** Expressed in conversation, intercourse, communication.
            - **Language Instinct:** Simplest social expression, greatest educational resource.
            - **Constructive Instinct:** Instinct of making; expressed in play, movement, make-believe, shaping materials.
            - **Investigation Instinct:** Grows from constructive and conversational impulses; child sees no difference between experimental science and carpentry.
            - **Expressive Instinct / Art Instinct:** Refinement and full manifestation of communicating/constructive instincts.
        - Schooling should foster these instincts to give purpose to a child's life.
- **3.3 Basic School Skills - The 3Rs and A C (Reading, Writing, Arithmetic, and Computer Skills):**
    - Children aged six have basic skills but need further teaching for perfection.
    - **3.3.1 Learning to Read:**
        - **Basic Intellectual Skill:** Foundation for all education and comprehension of other subjects.
        - **Progressive Steps:** Associating names with things, alphabet/letter sounds, common letter groups, simple text.
        - **Fluency:** By fifth/sixth grade, children are generally fluent.
        - **Good Reading Includes:** Clear pronunciation, voice modulation, recognizing emotional overtones, pauses, adequate speed.
        - **Development:** Can differ; poor comprehension can improve. Interest-based reading at home (by parents) can foster love for reading.
    - **3.3.2 Learning to Write:**
        - Developed by practice and organized plan.
        - **Dewey's View:** Spontaneous development through expression of thoughts, guided by realities.
        - **Russell's View:** Reading literature necessary for good linguistic skills, especially when primitive aesthetic impulses are lost; provides "habit of thought".
        - **Encouraging Good Writing:**
            - Exercises based on children's personal/classroom activities.

        - Understand different purposes of writing.
        - Teacher discusses purpose and provides points.
        - No interruption once writing begins.
        - Time for revision.
        - Display/publish all papers, avoid favoritism.
        - Plan reading/oral exercises around writing.
    - **3.3.3 Developing Math Skills (Arithmetic):**
        - "Bugbear" but natural introduction to **accuracy** (answers are right or wrong).
        - Difficulties should be carefully graded; not too much time at a stretch.
        - Formal geometry/algebra begin after middle childhood.
        - **Teaching Methods:** Concrete objects and abacus can improve computation skills.
    - **3.3.4 Computer Skills:**
        - Added as basic skill.
        - **Computer as Machine:** Children enjoy exploring. Formal teaching should not start before 14.
        - **Computer as Teaching Aid:** Source of tremendous information. Care needed regarding content access and time devoted.
        - **Impact:** Reinforces learning and provides exploration, but does not necessarily accelerate knowledge acquisition. Excessive use can cause physical/social problems.
- **3.4 Teaching for Knowledge:**
    - **Intelligence:** Includes actual knowledge and receptivity. Cannot be trained without imparting information.
    - **Subjects:**
        - **Geography:** About places, lands, etc. Should be taught through pictures, traveler tales, cinema, maps, and elementary information, fostering natural curiosity.
        - **History:** Introduced after geography (sense of time rudimentary). Start with illustrated stories of eminent personalities, visits to historical places. Aims to create a link between individual and humanity.
        - **Science Teaching:** Very basic at this stage (formal physics/chemistry later). Emphasize **"learning by doing," observation, and experimentation**. Contributes to imagination. Builds curiosity and aptitude for science.
        - **Language Teaching:** Start very young for perfect foreign language acquisition. Should be free from mental fatigue (games, puzzles, plays, acting).
        - **Literature:** Aims to train memory (poems, text) and develop sensitivity to language beauty. Should be associated with acting for lasting impression. Content chosen for pleasure.

- **3.5 Value Education:**
    - **Aim:** Developing **ideal character** with virtues such as:
        - **Vitality:** Physiological, pleasure in being alive, promotes objectivity, interest in outside world, hard work, safeguards against envy.
        - **Courage:** Absence of fear and power to control fear. Combination of self-respect and impersonal outlook to life.
        - **Sensitiveness:** Corrective of mere courage. Emotional reaction must be appropriate (pleasant behavior, sympathy).
        - **Intelligence:** Aptitude for acquiring knowledge (by exercise and information). Curiosity should be associated with observation, belief in knowledge possibility, patience, industry. Fosters open-mindedness and cooperation.
    - **Imparting Values:** Not through books. **Teacher's personality is important**; values transfer from teacher to disciple if practiced sincerely.
    - **Discipline:** Should grow out of and be relative to the aim (e.g., social cooperation vs. set lessons). School classroom can be like a busy workshop, with "disorder, confusion, bustle that results from activity." Deeper discipline lies in constructive work with purpose.
- **3.6 Other Subject Teaching:**
    - **3.6.1 Physical Education:** Completes education. Sports instill **cooperation** and allow physical growth. Includes health and hygiene (good habits, cleanliness, disease prevention).
    - **3.6.2 Teaching for Pleasure:**
        - **Dance and Music:** Good for body, aesthetic sense, and pleasure. Collective dance teaches cooperation. Singing (later than dancing) for beautiful songs.
    - **3.6.3 Educational Excursions:**
        - Make children familiar with surroundings, processes (agriculture, printing).
        - **Direct experience is crucial**; teaching from books alone is incomplete.
        - Maintain curiosity and willingness to learn more. Cinema and pictures can compensate if trips are not possible.
- **3.7 Let Us Sum Up:**
    - Middle childhood is for shaping abilities, directing curiosities, and strengthening emotional/intellectual aspects.
    - Schooling prepares minds for future life (healthy, happy, useful).
    - Teacher is vital for perfect mental and emotional development, acting as a leader to prepare children for specialized education.

Unit 4: Identification of Problems in School Children and Remedial Measures

- **4.0 Introduction:**

    - Not all children learn equally; **individual differences** exist.
    - **Exceptional Children / Children with Special Needs:** Children who find it difficult to learn like average children of the same age.
    - **Exceptionality:** Can be **positive** (gifted) or **negative** (learning disabilities, impairments).
    - **Special Education:** Education tailored for these children.
    - **Importance:** Early recognition of problems and timely remedial measures help children benefit from learning.
- **4.2 Exceptional Children:**

    - **Definition (Kirk):** A child deviating from the average in mental, sensory, neuromuscular, physical, social/emotional, communication, or multiple handicaps, requiring **modification of school practices or special education** to reach maximum capacity.
    - **Implications:** Cannot profit from regular school programs; need special methods and materials. Deviations can be positive or negative.
    - **Children at Risk:** No learning disability, but at risk of developing one (due to birth conditions, nurturing, or environment); experience learning, socialization, and maturational difficulties in classrooms.
    - **Special Education Definition: Individualized educational instruction** designed to meet unique needs of students with disabilities.
    - **Groups Served:** Learning disabled, speech/language impaired, mentally retarded, emotionally/behaviorally disturbed, physically impaired, autistic, deaf-blind, traumatic brain disordered, severely multiple handicapped, **Gifted and Talented**.
- **4.3 Learning Disabilities (LD):**

    - **Definition:** Syndrome in children of **normal or above intelligence** characterized by **specific difficulties** in learning to read (**dyslexia**), write (**dysgraphia**), and do grade-appropriate mathematics (**dyscalculia**).
    - **Characteristics:**
        - **Chronic condition of probable neurological origin**.
        - Varies in manifestation and severity; influences self-concept.
        - May persist into adulthood.
        - Intellectual functioning is normal; issues lie in **way of learning and perceptual systems**.
        - Behavioral problems (frustration-induced emotional problems) are secondary.
        - Boys more likely to be diagnosed.
        - May affect one area, not others.
        - Delay in developmental milestones.
        - **Behavioral/Affective:** Hyperactivity/hypoactivity, impulsivity, intense emotions.

        - **Attention Disorders:** Problems sustaining attention, easily distracted, short attention span.
        - **Perceptual Motor Impairment:** Poor auditory/visual discrimination, directional orientation issues, awkwardness, clumsiness, poor handwriting.
        - **Memory/Thinking Disorders:** Problems in short/long term memory, **metacognition** (ability to monitor/evaluate own actions), organizing, categorizing, planning.
        - **Specific Academic Problems:** Several years behind in reading, comprehension, fluency, spelling. Word, letter, number, sound reversals. Difficulties in oral/written language, recalling math facts, writing numbers, abstract math reasoning.
        - **Speech/Language Disorders:** Repeating sounds, stumbling, halting speech, pragmatic difficulties, word finding.
        - Some CNS signs/irregularities.
    - **4.3.2 Causes of LD:** Often complex interaction of factors.
        - **Environmental Model:** Poor learning environment, unstable families, disadvantaged environments, faulty school instruction. Improvement relies on environmental change.
        - **Brain Damage Model:** 20% of LD students have sustained brain damage or neurological damage (minimal brain dysfunction); injury before, during, or after birth.
        - **Organic and Biological Model:** Chemicals in food, neurotransmitter imbalances, vitamin deficiency (B complex), developmental or maturational unpreparedness.
        - **Genetic Model:** Inherited genetic influence may cause reading and language problems.
    - **4.3.3 Identification Process of LD:**
        - **Early detection/screening:** By class teacher observing behavioral/learning characteristics.
        - **Multidisciplinary Team:** Class teacher, school psychologist, clinical personnel determine disability degree.
        - **Assessments:** Achievement measurements (teacher-made tests, curriculum-based, standardized tests).
        - **Specific Tests: Wechsler Intelligence Scale for Children-III (WISC-III)** for cognitive abilities; **Woodcock-Johnson Psycho-Educational Battery-Revised (WJ-III)** for achievement (reading, writing, math); **Brigance Diagnostic Inventory of Basic Skills**.
    - **4.3.4 Remedial Programs for LD:**
        - **Individualized Teaching Program:** Specific instruction for specific needs (reading, writing, math).
        - **Direct Instruction (DI):** Highly structured, organized teaching, step-by-step with clear goals, feedback, corrections. Combined with

cognitive instruction (CI) emphasizing attending, responding, rehearsing, recalling, transferring information.
        - **Multisensory Instructional Strategies:** Learning by seeing, hearing, touching, movement.
        - **Study Skill Training / Meta-cognitive Skills:** Helps with note-taking, test prep, compositions, remembering materials.
        - **Social Skill Training:** Helps getting along with peers/adults.
        - **Inclusion Strategies:** Educating students with LD with non-disabled peers in general classrooms, using additional instructional resources.
        - **Other Methods:** Peer-mediated instruction, computer-assisted instruction.
        - **Teacher/Psychologist Role:** Sensitivity to problems, providing appropriate educational settings.
- **4.4 Mental Retardation (MR):**
    - **Definition:** Child scores **below 70 on standardized IQ test** and has **significant difficulties adapting to environment**.
    - **Classification:** Mild, moderate, severe, profound. Purely based on IQ scores.
    - **Causes:**
        - **Physical Causes:** Genetic/chromosomal disorders, brain damage (prenatal, birth, postnatal). Almost all severe/profound cases have physical causes.
        - **Cultural/Familial Causes:** No brain damage, but from deprived/dysfunctional environments.
    - **Impact:** Unable to make adequate adjustments due to limited intellectual/adaptive capacities.
    - **4.4.1 Identification Process of MR Children:**
        - **Assessments:** Intelligence and adaptive behaviors. Not based on one test.
        - **Comprehensive Testing:** Behavior observation (teacher), curriculum-based assessments, interviewing, standardized tests.
        - **IQ Tests:** Stanford-Binet Intelligence Scale, Revised Wechsler Intelligence Scale for Children-III, Kaufman Assessment Battery for Children (K-ABC).
        - **Adaptive Behavior Checklists: Vineland Adaptive Behaviour Scale** (self-help, locomotion, communication, occupation, socialization, self-direction); **Adaptive Behaviour Scale-Public School Version (ABS-PS)** (independent functioning, physical development, economic activity, language, numbers/time, vocational, self-direction, responsibility, socialization; also violent/destructive/antisocial/withdrawn/hyperactive behaviors).
    - **4.4.2 Remedial Measures for MR:**
        - **Early Intervention:** Programs providing guidance for families.
        - **Preschool Programs:** Focus on school readiness and socialization.

        - **Regular Classroom Programs:** For mild/moderate MR, individualized academic/functional programs, interaction with non-disabled peers.
        - **Resource Room Programs:** Remedial help.
        - **Self-contained Classroom Program:** For moderate/severe MR; segregated classroom, age/developmentally appropriate skills.
        - **Curriculum:** Designed for functional needs, accommodating traditional subjects (reading, arithmetic, science, health) to facilitate adjustment.
        - **Full Inclusion Programs and Individualized Education Programs (IEPs):** Used for better results.
        - **Behavior Therapy:** Decrease disruptive/inappropriate behaviors, emotional disturbances. Behavior modification techniques for attention, learning behaviors.
    - **4.4.3 Effective Teaching Strategies:**
        - Can use same instructional programs as for non-retarded, but require more time/effort.
        - **Focus on functional tasks** (e.g., reading signs for one, newspaper sections for another).
        - Teacher must know each student's ability to design individual programs. *__Inattentive:__ Careless mistakes, difficulty sustaining attention/listening/following through on tasks/organizing, avoiding strenuous tasks, losing materials, easily distracted, forgetting schedules.
        - **Hyperactive-Impulsive:** Fidgets, squirms, leaves seat inappropriately, runs/climbs inappropriately, avoids quiet activities, talks excessively, blurts answers, difficulty waiting/taking turns, butts into conversations/games.
        - Symptoms must be **persistent and extreme**, impairing adequate functioning.
    - **Etiology:** No agreement; viewed as medical disorder (neurological, brain structure, chemical imbalances) or affected by poor diet/parenting.
    - **Assessment/Identification Sequence:** Rating scales, family/student orientation, student/parent interviews, standardized tests (IQ, achievement), direct observations (school, community, home), medical evaluation, data integration, feedback/recommendations.
    - **Programs for Treatment/Education:**
        - **Medical Management:** Physician determines effectiveness of medication for hyperactivity.
        - **Psychological Counseling:** Helps student understand/cope with ADHD and negative effects; most effective with family involvement.
        - **Environment Arrangement:** Classroom/school structured to enhance student success (allow movement, effective work areas).
        - **Educational Planning:** Multidisciplinary team designs **IEPs**.
        - **Behavior Management Instruction:** Helps child recognize interfering behaviors; behavior modification techniques encourage appropriate activities.

- **4.6 Orthopedically Handicapped:**
    - **Impact:** Muscle and bone disabilities create school adjustment and educational achievement problems.
    - **Cerebral Palsy (CP):**
        - **Definition:** Disorder affecting movement/posture due to **brain damage** (often oxygen deprivation) occurring pre-, during, or post-birth.
        - **Characteristics:** Not a disease, not progressive, infectious, or hereditary.
        - **Types:**
            - **Spastic:** Stiff, tense, poorly coordinated movements.
            - **Athetoid:** Purposeless, uncontrolled involuntary movements, contorted purposeful, etc..
        - Organize furniture for easy movement (wheelchair, standing board).
        - Pace lessons for rest time.
        - Ensure directions are understood.
        - Assign necessary missed work, no extra drill.
        - Seek advice from counselors/mental health professionals. Support student, avoid complaints to family.
- **4.7 Hearing and Visual Impairments:**
    - **Detection:** Challenging for mild-to-moderate impairment, often misdiagnosed as MR.
    - **Hearing Impairments:**
        - More common than blindness. Severely affects comprehension and use of oral language; extensive training needed for language development.
        - **Educational Options:** Full-time/part-time regular class, special class, separate day/residential school.
        - **Approaches:**
            - **Oral/Aural:** Emphasize oral language, amplification (hearing aids), voice use.
            - **Manual:** Rely on sign language, non-oral means.
            - Audio-verbal training for residual hearing; speech reading, cued speech, sign language, finger spelling, total communication.
            - Technical devices: cochlear implants, computers.
        - **Teacher's Role:** Discuss lessons with interpreter. Face student when speaking. Use captioned videos/films. Wear contrasting clothing for sign language. Identify sound cues (bells, alarms). Be aware of extraneous noises. Allow free movement for speech-reading. Ensure well-lit classroom.
    - **4.7.1 Visual Impairments:**
        - **Blind:** Taught to read by **Braille**.
        - **Partially Sighted:** Use magnifying glass or large print books.
        - **Needs of Visually Impaired Students:** Concept development, academic skills, communication skills (handwriting, Braille, word processors,

note-taking), social/emotional skills, sensory motor skills, orientation/mobility (body image, spatial concepts, long cane, independent travel), daily living skills (personal hygiene, eating, money management), career partial vision, use large print with high contrast (e.g., black on yellow).

- **4.8 Gifted and Talented Children:**
    - **Definition (Terman):** Demonstrate achievement or potential in:
        - General intellectual ability (high IQ).
        - Specific academic aptitude (excellence in math/science).
        - Creative/productive thinking (discover new things, new alternatives).
        - Leadership ability (solve problems).
        - Visual/performing arts (art, music, dance, drama).
        - Psychomotor ability (excellence in sports).
    - From all societal levels, races, ethnic groups. Process information differently.
    - **Challenges:** May have social adjustment problems, emotional disturbances, boredom with regular curriculum. Other children may perceive as "showoff".
    - **Importance:** If talent not nurtured, it's a loss to society and individual.
    - **Options for Education:** Early admission, acceleration, **enrichment**.
        - **Enrichment:** Attempts to **broaden knowledge, add depth, detail, and challenges to curriculum**. Involves independent study, small projects. Needs clear purpose and outcomes, well-planned and organized.
    - **4.8.1 Role of the Teacher:**
        - Provide resource materials (books, computers).
        - Allow expression of interests, display research.
        - Encourage divergent thinkers, guide to find more information [40 educating them in the same classroom/school settings as their peers.
    - **Goal:** Equalizing and mainstreaming, eradicating exclusion.
    - **Approach:** Refining instructional procedures to suit each student's needs. Needs fulfilled by individualized instruction, providing equipment/materials.
    - **Teacher's Role:** Must be sensitive to abilities/disabilities and trained to attend to various problems.

Block 3: Adolescent Development

Unit 1: Physical Changes

- **1.0 Introduction & 1.2 Adolescent Development: Meaning and Definition**

    - Adolescence is a **transitional phase from child to adult**, generally extending from **12 to 19 years of age**.
    - It is broadly categorized into three stages: **early, middle, and late adolescence**.
    - Teenagers experience **physical milestones** as they move from childhood to adolescence, including an **increase in body size and sexual maturation (puberty)**.
    - This transition involves **biological (pubertal), psychological, and social changes**.
    - Parents may find this period confusing or "stormy".
- **1.2.2 Early Adolescence (12-14 years)**

    - A **contradictory phase** where the child is not yet mature but no longer a child.
    - They insist on independence, but parents may not grant much freedom.
    - **Drastic physical changes** can be a **source of irritation**.
    - Some may feel **inferior** if physically underdeveloped, fearing ridicule.
    - Marked by **confusion, hesitation, and a search for identity** ("Who am I? What am I?") because they look like adults but are mentally still children.
- **1.2.3 Middle Adolescence (14-17 years)**

    - A phase of **transformation** with significant physical, mental, cognitive, and sexual changes.
    - Most girls cross their **puberty stage**, while boys are still maturing physically.
    - Young individuals are **most concerned about their look, body, and appearance**.
    - Developing their **unique personality and opinions**.
    - **Friends play a pivotal role** in maintaining identity within the peer group.
    - **Competitiveness** becomes a major priority.
    - Many analyze inner turbulence by **writing diaries and journals**.
    - More capable of **setting goals**, though often too high, leading to frustration and disappointment.
    - **Withdrawal from parents** (reduced dependency) and attempts to make their own decisions.
- **1.2.4 Late Adolescence (17-19 years)**

    - Closer to adulthood, developing a **firm identity and more stable interests**.
    - Attain **greater emotional stability** and a **more developed sense of humor**.
    - Able to **delay gratification** and **think ideas through**.
    - Learn to **express feelings in words, compromise, and make independent decisions**.
    - Show **greater concern for others** and become more **self-reliant**.
    - Start **finalizing thoughts about future roles**.
    - Table 1 provides **reference body weights and heights** for boys and girls in this age group.

- **1.3 Physical Development & 1.3.1 Puberty**
    - Adolescence is characterized by **dramatic physical changes** moving from childhood to physical maturity.
    - Normal growth includes an **increase in body size and sexual maturation (puberty)**.
    - **Timing and speed of changes vary** due to heredity and environment.
    - Boys and girls reach **adult height and weight**.
    - Adolescence is a time for **growth spurts and pubertal changes**; growth can occur in spurts.
    - **Puberty:** The stage where one becomes **biologically and sexually mature**. A 1-3 year process of hormonal and physical change. Girls usually enter it about a year earlier than boys. It's the entry into adolescence, bringing changes in body, emotions, attitude, values, intellect, and relationships.
    - Puberty involves **hormonal changes**, with the pituitary gland increasing **follicle-stimulating hormone (FSH)**, which activates ovaries to produce **estrogen** in girls and causes **sperm to develop** in boys.
    - Often described as a period of **turbulence and trials** due to developing sex drive, mood swings, new relationships, and pressures.
    - Adolescents often feel they **lack adequate information** and adults don't understand them, leading to **worry, fear, and insecurity**. These feelings are normal.
    - Anxiety about the changing physical body is common. Understanding these changes helps.
    - Changes are interconnected and proceed smoothly internally, despite external confusion.
    - In the brain, the pituitary gland releases hormones (testosterone in boys, estrogen in girls) that lead to body changes.
- **Adolescent Males: Physical Changes**
    - **Age of Puberty:** 9.5 to 14 years. Changes begin between 12 and 14 years.
    - **First Pubertal Change: Enlargement of the testicles**. Testicles may double or quadruple in size, and scrotal skin changes color and texture.
    - **Penis Enlargement:** Begins about one year after testicles enlarge. Gets longer then broader; final size reached around 15-16 years.
    - **Appearance of Pubic Hair:** Around 13.5 years. Hair also grows under arms, on face, and legs. Face hair starts fine, then becomes thicker with shaving.
    - **Voice Change:** Voice gets deeper, may "crack" temporarily, usually around 15 years. Caused by larynx expansion due to testosterone.
    - **Acne:** Around 15 years, due to increased oily skin and sweating. Pimples, spots, blackheads can appear on face, neck, chest, back from sebaceous glands blocking pores.

    - **Nocturnal Emissions (Wet Dreams):** Around 14 years. Involuntary release of semen during sleep due to semen build-up. Normal part of puberty that stops with age.
    - **Height Increase:** 4 to 12 inches. Growth spurt around 13-14 years, ending 16-17 years. Boys grow later and longer than girls.
    - **Body Size:** Feet, arms, legs, hands may grow faster than the rest of the body, causing a feeling of **clumsiness**.
    - **Breast Swelling:** Some boys may experience temporary breast swelling due to hormonal changes.
    - **Erections:** Penis becomes hard and erect due to hormonal changes, can occur spontaneously or with sexual thoughts. Nerve impulses fill penis with blood, making it stiff and larger.
    - **Ejaculation:** Release of semen (sperm + bodily fluids) from penis. Essential for reproduction, leads to sexual pleasure. Not always voluntary. Muscles close off bladder during ejaculation.
    - **Testosterone:** Male hormone produced by testicles, becomes active after 13-14 years, transforming boy to man.
    - **External Changes:** Most changes in boys are visible, unlike girls.
    - **Body Odour:** Sweat glands become more active, creating unpleasant smell when sweat contacts skin bacteria.
    - **Sexual Feelings:** Thoughts and fantasies become more frequent, awareness of sexual stimuli. Normal.
- **Adolescent Females: Physical Changes**
    - **Age of Puberty:** 8 to 13 years. Pubertal changes usually begin before boys of the same age.
    - **First Pubertal Change: Breast development**. Breasts enlarge with fat deposit, become soft and rounded.
    - **Pubic Hair Development:** Shortly after breast development. Hair is V-shaped, thicker, curlier, coarser than head hair.
    - **Hair Under Arms:** Around 12 years of age. Also increased hair growth in pubic area and legs.
    - **Menstrual Periods:** 10 to 16.5 years of age. Begins when ovaries release an egg. If egg not fertilized, uterine lining shed as fluid (bloody, usually monthly). After menstruation, a girl can get pregnant.
    - **Height Increase:** 2 to 10 inches. Greatest increase around 10-11 years. Growth usually stops between 16-18 years.
    - **Body Shape Changes:** Increase in height/weight, **hips may get wider**, increase in fat in buttocks, legs, and stomach. These are normal.
    - **Clumsiness:** Feet, arms, legs, hands may grow in advance of the body, leading to a feeling of clumsiness.
    - **Oily Skin and Acne:** Increased oily skin and sweating due to hormones; acne may develop. Pimples from sebaceous glands blocking pores.
    - **Invisible Changes (Reproductive Organs):**

        - **Ovaries:** Two, almond-sized, contain tiny egg cells. Release an egg (ovum) monthly, sometimes more. Produce **estrogen** (makes girl's body woman-like, enables children).
        - **Uterus (Womb):** Upside-down pear-shaped, hollow, made of smooth muscles. Where baby grows, expands during pregnancy. Narrows into cervix, which opens into vagina.
        - **Fallopian Tubes (Uterine Tubes/Oviducts):** Trumpet-shaped, near ovaries, open into uterus. Pick up eggs and push into uterus (takes 3-7 days).
        - **Vagina:** Passageway/muscular tube from cervix to external genitals. Opening between urinary opening and anus. Passageway for menstrual flow, baby at birth, sexual intercourse. Male sperm travel through it.
        - **Clitoris:** Small cylindrical body above urinary opening, provides sexual pleasure.
        - **Hymen:** Porous membrane near vaginal opening, may tear during first penetrative sexual act with slight bleeding/pain.
    - **Visible Changes:** Growth spurt (height, pelvic bones, hips widening), breast development, hair growth (arms, underarms, legs, upper lips, pubic area), body odour, pimples, sexual feelings.
    - **Sexual Feelings:** May start to feel sexually aroused, spend time fantasizing about love and sex. Normal due to changing hormones.

Unit 2: Cognitive Changes

- **2.0 Introduction & 2.2 Cognitive Development: Meaning and Definition**
    - **Cognition** refers to processes by which **sensory input is transformed, reduced, elaborated, stored, recovered, and used**. It also includes processes operating without stimulation, like images.
    - **Cognitive psychology** investigates internal mental processes like visual processing, memory, thinking, learning, feeling, problem solving, and language. It uses scientific methods and accepts internal mental states.
    - **Cognitive development** refers to the development of the **ability to think and reason**. It involves how a person perceives, thinks, and understands their world through genetic and learned factors.
    - It takes a **fast pace during adolescence**. Teenagers accumulate general knowledge and apply learned concepts to new tasks.
    - Adolescence marks the beginning of more complex thinking processes called **formal logical operations**.
- **2.2.1 Cognitive Development in Adolescence**
    - Adolescence (11-12 to 19-21 years) involves the development of **abstract thinking** (thinking about possibilities), **reasoning from known principles** (new ideas/questions), considering **multiple points of view**, comparing/debating ideas, and developing the **ability to think about thinking** (metacognition).

    - A sense of **ego and personal uniqueness** develops, leading to a belief that "no one can really understand her/him".
- **2.2.2 Early Adolescence (Cognitive)**
    - Able to use **more complex thinking**.
    - Focused on **personal decision-making** in school and home.
    - Begin to demonstrate **formal logical operations** in school work.
    - Begin to **question authority and society standards**.
    - Form and verbalize their own thoughts on topics related to their lives (e.g., sports, groups, appearance, parental rules).
- **2.2.3 Middle Adolescence (Cognitive)**
    - Focus expands to include more **philosophical and futuristic concerns**.
    - Often **question and analyze more extensively**.
    - Think about and begin to form their **own code of ethics**.
    - Think about different possibilities and begin to develop their **own identity**.
    - Systematically consider **possible future goals** and **make their own plans**.
    - Begin to think on a **long-term basis**.
    - Systematic thinking influences relationships with others.
- **2.2.4 Late Adolescence (Cognitive)**
    - Complex thinking focuses on **less self-centered concepts** and personal decision-making.
    - Start thinking more about **global concepts** like justice, history, politics, patriotism.
    - Often develop **idealistic views** on specific topics.
    - **Debate and discuss a great deal**, sometimes showing **intolerance to opposing views**.
    - Begin to focus thinking on **career decisions** and their **emerging role in adult society**.
    - Adolescents become capable of **logical thought**, though may not always demonstrate it.
    - Understanding of how they and others think continues to develop.
    - Progression of understanding knowledge: Realistic approach (knowledge as definite facts) -> Awareness of expert disagreement (relativistic approach) -> Defended realism (facts vs. opinions) -> Dogmatism-skepticism (blind faith vs. doubting everything) -> Post-skeptical rationalism (no absolute truths, but better reasons for views).
    - Cognitive development continues throughout adolescence, leading to **more mature modes of thought**.
- **2.3 Piaget's Theoretical Approach on Cognitive Development**
    - Swiss psychologist Jean Piaget (1896-1980) is the most influential theorist.
    - Proposed a more active role for children in knowledge acquisition.

- Child's knowledge composed of **schemas** (basic units of knowledge to organize/understand experiences).
- Theory is a comprehensive theory about the **nature and development of human intelligence**.
- It is a **developmental stage theory**, dealing with how humans acquire, construct, and use knowledge. Cognitive development is central, and language is contingent on it.
- Discusses **four distinct, universal stages**, each with increasingly sophisticated thought levels, occurring in the same order, building on previous learning.
- **Stages:**
  - **Sensorimotor Stage (infancy):** Intelligence through motor activity (no symbols), physical interaction-based knowledge, object permanence, some language by end.
  - **Pre-operational Stage (toddlerhood/early childhood):** Intelligence through symbols, language matures, memory/imagination, non-logical/non-reversible thinking, egocentric thought predominates.
  - **Concrete Operational Stage (elementary/early adolescence):** Seven types of conservation, logical/systematic manipulation of concrete objects, operational thinking develops, egocentric thought diminishes.
  - **Formal Operational Stage (adolescence and adulthood):**
    1. Adolescents (12-19) are/should be in this stage.
    2. Intelligence through **logical use of symbols related to abstract concepts**.
    3. Early in this period, there is a return to **egocentric thought**.
    4. Increased independence for **thinking through problems/situations**.
    5. Ability to **understand pure abstractions** (philosophy, higher math).
    6. Learn and apply general information to specific situations; learn skills for occupation.
    7. Adolescents think in ways that are **more advanced, efficient, and generally more complex** than children.
- **Five ways thinking advances during Formal Operations (Reiteration of 2.2.1):**
  - **Think about possibilities:** Hypothesizing instead of limiting to reality.
  - **Think about abstract ideas:** Comprehend higher-order logic (puns, proverbs, metaphors), apply reasoning to social/ideological matters (relationships, politics, philosophy, religion, morality).
  - **Metacognition:** Thinking about thinking itself, leading to introspection and self-consciousness. This can lead to **adolescent egocentrism** (preoccupation with self).
  - **Multidimensional thinking:** Seeing things through complicated lenses, describing self/others in differentiated terms, looking at problems from multiple perspectives. Leads to more sophisticated relationships.

        - **Relativistic thinking:** Seeing things as relative rather than absolute (questioning assertions, less acceptance of absolute truths). Can be exasperating to parents.
- **Egocentrism in Adolescence**
    - An important aspect of psychosocial development.
    - Elkind (1967): Includes a **belief system that makes adolescents consider themselves special and unique**.
    - Characterized by an **imaginary audience** (belief that others, especially peers, observe and comment on their activities) and **increased self-consciousness**.
    - Extremely conscious of what others think of their appearance and everything related to themselves. This perception contributes to the **development of self-confidence**.
- **2.4 Information Processing Perspective**
    - **2.4.1 Information Processing:**
        - **Definition:** Change (processing) of information detectable by an observer. Describes everything that happens in the universe as a change.
        - Human information processing: Acquiring, retaining, and using information.
        - **Input:** Information from the outside world into **sensory registers**. Most is unconscious; conscious awareness requires directed attention.
        - **Working Memory (Short-Term Memory):** Where conscious attention places information.
            1. **Limited capacity:** Can attend to only about **seven items at a time**.
            2. Actions: 1) continuously **rehearse** to keep it, 2) move to **long-term memory**, or 3) **forget** it.
        - **Long-Term Memory:** Stores information for a **long time, perhaps permanently**. No restrictive limit on amount.
        - **Retrieval:** Process of moving information from long-term memory back to working memory to use it.
        - **Analogy:** Executive managing a business (mail/calls as short-term, wastebasket as forgetting, filing as long-term storage, retrieving files for new info).
    - **2.4.2 Information Processing Theory:**
        - Associated with high-speed computers (1950s).
        - Computer-oriented models provided insight into how human mind receives, stores, retrieves, and uses information.
        - Ended behaviorism's dominance, focusing on **innate mental capacities** rather than conditioned behavior.

        - Reestablished internal thought processes as legitimate scientific inquiry area.
        - Approach to cognitive development that analyzes the **sequence of events in a person's mind when receiving new information**. Analysis of how humans learn new things.
        - Fixed pattern of events: information analyzed, benchmarked, then stored.
        - Claims human mind is **similar to computers** in processing/analysis.
        - Sensory perceptors like computer hardware; mindset/rules/strategies like software. Can be enhanced by altering these.
    - **2.4.3 Information Processing Model:**

        - Fixed structure with four parts:
            1. **The Store Model:** Information stored in processing units/channels (sensory register, short-term memory, long-term memory).
            2. **The Sensory Register:** Receives all information, stores temporarily or permanently.
            3. **Short-Term Memory:** Information stored temporarily; discarded or transferred to long-term memory.
            4. **Long-Term Memory:** Information permanently stored, retrieved when needed.
        - **Four Main Beliefs:**
            1. **Thinking involves:** Perceiving, encoding, representing, storing, retrieving information, and responding to memory constraints.
            2. **Development driven by change mechanisms:** Encoding, strategy construction, automatization, generalization. Children encode critical info, use it with prior knowledge.
            3. **Development driven by self-modification:** Children actively modify responses to new problems using acquired knowledge/strategies.
            4. **Careful task analysis:** Child's development level and task nature constrain performance. Extra/misleading info can confuse.
- **2.5 School Performance and Cognitive Development**

    - **Reasons for Underperformance:** Lack of motivation, home/peer problems, poor work habits/study skills, emotional/behavioral problems, learning disabilities (dyslexia), ADHD, mental retardation/below average intelligence, other medical problems (anxiety, depression).
    - **Achievement Motivation:** Adolescents' beliefs, values, goals. Decline in motivation during early and middle adolescence.
    - **Early adolescents** have **lower perceptions of competence** for school subjects. Valuing of school subjects often declines, especially in middle school transition. Intrinsic motivation for learning decreases.
    - **Motivational problems** become more central in early adolescence.
    - **Role of Counselors and Teachers:**

        - Identify at-risk children and foster motivation.
        - Ease transition to middle school.
        - Accomplished through: **cooperation** (counselors, teachers, administrators), **parent involvement**, **programs/curricula** for academic/social/career development, **responsive counseling** for personal issues, **cultural awareness**, and **advocacy** for diverse needs.
        - Focus on **developmental issues of early adolescents** is important.

Unit 3: Identity, Self Concept, Self Esteem, Peer Group Relationship

- **3.0 Introduction & 3.2 Identity in Adolescence**
    - Adolescence is a period of **exploratory self-analysis and self-evaluation** leading to a **cohesive and integrative sense of self or identity**.
    - Identity search is affected by the **social world: peers, parents, schools, and neighborhoods**.
    - Identity formation involves **negotiation of activities and relationships** (school, social, career interests) and **exploration of different roles**.
    - Gender, ethnicity, sexual orientation are important to developing identity.
    - **Identity** is a **new way of thinking about oneself** that emerges during adolescence. It involves a **sense of self-unity**, continuity over time, and uniqueness.
    - According to Erikson's psychosocial model, identity must be **perceived by the individual** and **recognized/confirmed by others**.
    - Process involves "integrating into a coherent whole one's past experiences, ongoing personal changes, and society's demands and expectations for one's future".
    - Identity development begins in infancy (discovery of self, "me," "I," "mine").
    - Childhood: self-awareness grows, descriptions include observable characteristics, preferences, competencies (preschoolers) and feelings, social fit (school-aged).
    - Erikson identified the goal of adolescence as **achieving a coherent identity and avoiding identity confusion**. Identity is multidimensional (physical, sexual, occupational, religious, ethnic).
    - Two steps in identity development: 1) **breaking away from childhood beliefs to explore alternatives**, and 2) **making a commitment** to an individual identity in that area.
    - **Foreclosure status:** Commitment made **without exploring alternatives**.
    - Identity achievement in adolescence forms the **basis for adult expectations and goals**.
    - Lifespan construct: Integration of past, present, and culture linking adolescent identity to adult self.
- **3.3 Identity Crisis**
    - Coined by psychologist **Erik Erikson**.

- A time of **intensive analysis and exploration of different ways of looking at oneself**.
- Involves losing a sense of **personal sameness and historical continuity**.
- Occurs during the teenage years, where people struggle between **identity versus role confusion**.
- Erikson believed adolescents need to **try on a variety of roles** and "test extremes before settling on a considered course".
- Adolescents may **reject parents and their values** to make a clean break from childhood and form their own identity.
- They seek **role models** and turn to **peer groups** for belonging. This can lead to conformity within peer groups.

- **3.3.1 Marcia's Identity Statuses**

  - James Marcia (1966) refined Erikson's work, viewing identity as a **structure of beliefs, abilities, and past experiences** regarding the self.
  - A well-developed identity provides a **sense of strengths, weaknesses, and uniqueness**.
  - Less developed identity -> confusion about distinctiveness, reliance on external evaluation.
  - Identity is **dynamic**, not static.
  - Marcia identified four **identity statuses** (not stages) based on **crisis (exploration)** and **commitment** in domains like occupation, religion, and politics.
    - **Foreclosure:** Individuals have made **commitments** (e.g., career) but **have NOT experienced an identity crisis** (i.e., not explored options). They conform to others' expectations.
    - **Diffusion:** The young person has **NOT made a commitment** and may or may not have experienced a crisis. They appear to have given up on identity formation.
    - **Moratorium:** Individuals are **actively exploring alternative commitments** and **are experiencing an identity crisis**, but have not yet made a decision. They are moving towards identity formation.
    - **Achievement:** The individual has **experienced an identity crisis (explored alternatives) and has made commitments** necessary for a clear sense of identity.

- **3.4 Social Development**

  - Parental influences decrease, **peer influences increase**.
  - Identity search through interactions with peers, teachers, neighborhood adults.
  - **Same-sex hero worship** common; impressed by idealism of heroes and social workers.
  - Enjoy doing work for others and serving the community.
  - May imitate film heroes. All these efforts contribute to self-identity.
  - Adolescents experience **many moods and mood swings**.

    - Advancing reasoning skills allow them to ponder options and possibilities in social situations, think more logically and hypothetically.
- **3.5 Self Concept and Self Esteem in Adolescence**
    - **Self-Concept:**
        - **Accumulation of knowledge about the self** (beliefs about personality traits, physical characteristics, abilities, values, goals, roles).
        - Becomes **more abstract, complex, and hierarchically organized** into cognitive mental representations (self-schemas) in adolescence.
        - **Sum total of a being's knowledge and understanding of his or her self**.
        - Different from self-consciousness (awareness of one's self).
        - Components: physical, psychological, social attributes, influenced by attitudes, habits, beliefs, ideas.
        - Refers to **self-evaluation or self-perception**, representing the sum of beliefs about one's attributes.
        - Reflects how an adolescent evaluates self in domains where success is important. Can be **positive in some domains and negative in others**.
        - Categories: **Personal self-concept** (facts/opinions about oneself), **Social self-concept** (perceptions of how others regard one), **Self-ideals** (what one would like to be).
        - Often **positively correlated with academic performance**, appearing as a consequence rather than a cause of high achievement.
        - Low self-concept often associated with depression, but evidence is weak. Weak risk factor for smoking in girls, little support for link with alcohol/illegal drugs.
        - **Dynamic**, problems can lower it, and low self-concept can cause problems. High academic self-concept associated with positive academic performance; high physical self-concept with increased physical activity.
    - **Self-Esteem:**
        - A student's **overall evaluation of him- or herself**, including feelings of general happiness and satisfaction.
        - Differs from self-concept (which is perceptions of competence in domains).
        - **Important for success in life**; building it in adolescence can last a lifetime.
        - Many teenagers have **low self-esteem**.
        - **Major Reasons for Low Self-Esteem:**
            - **Heredity:** Introverted parents may lead to children with low self-esteem.
            - **Living Conditions:** Poor environment, lack of attention/opportunities.

        - **Lack of Proper Education:** Problems interacting with educated peers.
        - **Physiological Changes:** Difficulty coping with major physical changes and hormonal shifts during puberty.
        - **Societal Implications:** Restrictions on behaviors and attitudes (e.g., mingling with opposite gender) can create fear.
        - **Fear About Future:** Depression from unemployment, dating problems, insecurity, financial background.
        - **Diseases and Other Physical Ailments:** Feeling like "debris in the world," leading to isolation and agitation.
    - If not addressed, low self-esteem can lead to physical and mental ailments.
    - **Signs of Negative Self-Concept:** Doing poorly in school, few friends, putting oneself/others down, rejecting compliments, teasing, excessive anger/jealousy, appearing conceited, hesitating to try new things.
    - **Strategies to Improve:** Praise accomplishments, praise effort, encourage improvement, refrain from negative feedback.

Unit 4: Challenges and Issues in Adolescent Development

- **4.0 Introduction & 4.2 Challenges of Adolescence**
    - Adolescence is a phase of life beginning in biology and ending in society.
    - Defined as the period when most biological, cognitive, psychological, and social characteristics change from childlike to adult-like.
    - A **dramatic challenge** requiring adjustment to changes in self, family, and peer group.
    - Includes institutional changes (school transitions, high school to work/university/childrearing).
    - Time of excitement, anxiety, happiness, troubles, discovery, bewilderment, breaks with past, links with future. Can be confusing for adolescents, parents, and other adults.
    - Often romanticized as "stormy years".
    - Challenges are fast, everywhere, and hard to keep up with (body changes, thinking process, social life).
- **Identity Formation Challenges:**
    - Adolescents yearn to develop a **unique and independent identity, separate from their parents**.
    - They challenge parental authority, disobey rules, criticize values, discard suggestions.
    - Prefer to spend more time with peers and give them credit.
    - Yearn to be accepted by peers, going out of their way to gain approval, hiding weaknesses, exaggerating strengths.

    - Shun social interaction with parents and other adults. This preference can lead to anxiety and depression.
    - Parents should play a **highly supportive role**, understanding the complex and conflicting needs of teenagers.
- **Challenges for Adolescents with Autism:**

    - Bring a "special flavor" determined by **interest, avoidance, and insight**.
    - **Level of Interest:** Moderate to severe autism -> little/no interest in others, may be indifferent to peers. Less severe -> interest usually increases.
    - **Level of Avoidance:** Social anxiety can lead to avoidance, appearing as disinterest. Differentiating anxiety from genuine disinterest is important, as anxiety is more treatable.
    - **Level of Insight:** For those who interact, quality depends on insight into social disability. Lack of awareness -> rude, insulting, boring; missing criticism/sarcasm. Better insight -> motivated to learn social cues, work through sense of loss.
- **4.2.1 Coping with the Loss of Normalcy (Autism)**

    - Most children with autism realize they are "not quite like others" during adolescence.
    - Factors facilitating this realization: higher interest in others, higher insight into social difficulties, higher IQ.
    - Adolescents need to **deal with this loss** like any other loss.
    - Caregivers' job made easier by considering coping in stages (though not always linear): **Anger, Denial, Depression, Acceptance, Adaptation**.
    - **Parents' Role:** Help the youngster face the problem, handle it calmly, encourage talking about anger/frustration. Listen patiently without trivializing or exaggerating the problem. Offer counseling if hesitating to open up.
    - **Recognizing Depression:** Symptoms like sadness, irritability, sleep/eating disturbances, self-deprecation, suicidal remarks, loss of interest, withdrawal, self-blame. Seek immediate psychiatric help if 5+ symptoms present weekly.
    - **Leveraging Strengths:** Many excel in one or two subjects, love to talk about them. Parents should engage them creatively to boost self-esteem.
    - **Finding Identity in Autism:** Some resolve loss by embracing autism identity, connecting with other youths, educating peers, setting up websites/chat rooms, advocating for understanding/treatment. Encouraging this can be a great antidepressant.
- **4.2.2 Acknowledging Sexuality**

    - Adolescents with autism develop physiologically and sexually at the **same pace as their peers**.
    - Parents worry about **misunderstood behavior** or child being **taken advantage of** (pregnancy, STDs).
    - Concerns: child showing no interest, or inappropriate behaviors (touching private parts/others in public, stripping, masturbating, staring, inappropriate talk).

    - Parents often feel **uncomfortable talking about sex**, hoping it resolves itself or someone else handles it. May believe ignorance prevents sexual activity.
    - **Parents should address sexuality directly**, set aside time to talk, not just react to incidents.
    - Ask about child's knowledge, desires, worries. Inform them, set **realistic but firm limits** on inappropriate behavior.
    - Parental comfort conveys that sexual feelings are OK to have and talk about.
    - Seek help from other parents, school, or professionals who provide individualized sexuality assessment, sex education, and behavior modification techniques.
- **4.2.3 Change in the Point of View**

    - Adolescents manifest **clear sexual identity** and are concerned with **serious relationships**.
    - Capable of **tender and sensual love**.
    - **Self-esteem and personal dignity** become important; they desire to be **respected and treated as adults**.
    - Can offer useful insight, set goals, and follow through.
    - Learn **self-regulation** and accept social institutions/cultural traditions more easily.
    - Mental and emotional problems are often treatable with professional help.
- **4.2.4 High-risk Behaviours**

    - Late adolescence is a unique transition period.
    - Generally a healthy group, but may develop **chronic medical problems**.
    - **Eating disorders** are unique to this group (concerned about looks, starve themselves to avoid obesity).
    - Tend to develop **high-risk behaviors**: experimenting with drugs, alcohol, smoking, adventurous sports.
    - Behaviors often **challenge existing norms** and systems, taking risks to convince self/peers/parents they are grown up.

Block 4: Adulthood and Ageing

Unit 1: Physical Changes (Early Adulthood, Middle Age, Old Age)

- **1.0 Introduction**

    - Lifespan is divided into recognized stages by societies.
    - Adulthood includes: **young adulthood (20-40), middle adulthood (40-65), and later adulthood/old age (65+)**.
    - These stages are marked by physical, psychological, and social milestones.
- **1.2 Early Adulthood (20-40 years)**

    - Characterized by being **vibrant, active, and healthy**.
    - Focus on **friendship, romance, child-bearing, and careers**.
    - One of the hardest times after teenage years, involves **self-search and preparation**.
    - New sense of **independence and freedom**, with added personal responsibility.
    - Becomes financially less dependent on parents.
    - Major developmental tasks: **career choice**, settling down, starting a family, forming close relationships with professional peers and community members.
    - Responsibilities are two-fold: towards self and society.
    - Challenges: establishing financial/emotional independence, entering marital relationships.
    - Crises: **unemployment and marital discord**.
    - Erikson: crisis of **intimacy versus isolation** (ability to form deep intimate relationships).
    - Freud: time for **work and love**.
- **1.2.1 Physical Changes in Early Adulthood**
    - May continue to add a bit of **height and weight** to teenage frame.
    - **Hormonal changes** continue: beards may grow thicker, voice deeper/richer in men.
    - Women may gain weight and complete full **breast development during child-bearing**.
    - Females reach adult height by age 18; most males by 21 (some continue into early 20s).
    - **Muscles continue to gain mass** (especially males); both genders add body fat.
    - Average weight gain for men and women is about 15 pounds.
- **1.3 Middle Age/Middle Adulthood (40-65 years)**
    - Important challenge: develop **concern for welfare of future generations** and **contribute to the world** through family and work.
    - Erikson: **generativity versus self-absorption** (meaningful contribution vs. preoccupation with selfish needs).
    - Characterized by **competence, maturity, responsibility, and stability**.
    - Time to enjoy job success, family, social life, and children's success.
    - Increased focus on **health, aging parents, leisure time, and plans for old age**.
    - Period of **creativity and significant contribution to society** (best works of scientists, writers, artists).
- **1.3.1 Physical Changes in Middle Adulthood**
    - Individuals vary in rate of change, but all notice signs of **deterioration**.
    - **"Age thirty transition":** Reappraisal of choices in career and social relations.
    - **Vision:**
        - **Presbyopia:** Decline in near vision (lens less elastic) usually in 40s; reading glasses may be needed.

        - Increased sensitivity to glare.
        - Longer time to adapt to changes in illumination (50s).
    - **Appearance:**
        - **Loss of elasticity in skin**, especially face, resulting in **lines and wrinkles** (first signs of aging).
        - Most individuals get a little **shorter**.
        - **Hair starts graying**, skin wrinkling, bodies sagging, teeth yellowing.
        - Some strive to look younger with plastic surgery, hair dye, wigs, exercise, vitamins.
    - **Body Composition:**
        - Men typically gain weight in the abdominal region.
        - Women gain weight in hips and thighs.
        - **Strength and flexibility wane** in both genders.
        - Men show greater concern for health, strength, power, and sexual potency.
    - **Menopause (Women):** Occurs between **45 and 50 years**. Hormonal changes lead to loss of reproductive ability. May be accompanied by distressing physical and psychological symptoms.
- **1.4 Old Age/Late Adulthood (65+ years)**
    - Period of life **after 65 years**, nearing or surpassing average human lifespan.
    - Marked by **growing old**, resulting from failure of body cells to function normally or produce new ones.
    - Considered old due to changes in activities or social roles (e.g., grand-parenting, retirement).
    - Have **limited regenerative abilities** and are more prone to **disease, syndromes, and sickness**.
    - Results in significant physical, psychological, and cognitive changes (cardio-vascular, digestive malfunctioning, depression, impaired memory).
    - **Functional age:** Actual competence and performance, regardless of chronological age.
        - **Young-old:** Physically young for their years.
        - **Old-old:** Frail and show signs of decline.
- **1.4.1 Physical Changes in Old Age**
    - Natural changes not due to disease include sensory, digestion, circulation, and sexuality.
    - **External Changes (obvious):**
        - **Hair:** Most becomes distinctly gray/white, may thin out.
        - **Skin:** Less elastic, more wrinkled, dry, thin. Wrinkles partly from loss of fatty tissue. Tears and breaks more easily, increasing injury/infection risk.
    - **Sensory Changes:** Often diminished vision, hearing, taste, smell, touch.

        - **Hearing:** About 30% over 60 have impairment; 50% over 85. Trouble hearing high tones, making out sounds with background interference. Can lead to frustration, embarrassment, withdrawal.
        - **Vision:** Changes begin late 30s/early 40s. Difficulty focusing on near objects (presbyopia), reduced peripheral vision, slower adaptation to light changes. Degeneration of eye muscles, clouding/yellowing of lens. Red, yellow, orange easier to see than blue/green.
            - **Cataracts:** Cloudy lens areas, blur vision, can cause blindness.
            - **Macular degeneration:** Breakdown of light-sensitive cells in retina, blurry central vision, eventual blindness. Diet high in antioxidants can delay.
            - Compromised depth perception and visual acuity. Driving may be curtailed. Higher risk of stumbling/falls.
        - **Taste and Smell:** Minor loss in taste sensitivity after 70, some smell loss. Complaints often due to loneliness, inability to cook, dental problems, limited budget.
        - **Touch:** Reduced sensitivity to heat sources. Greater sensitivity to cool temperatures/drafts (due to decline in sweat gland activity, poorer circulation, thinning skin). Important for emotional well-being.
    - **Internal Changes:**
        - **Bones and Muscles:** Vulnerable to broken bones. Joints stiffen, ligaments lose elasticity (hand/foot pain). Regular exercise helps mobility. Falls often result in injury, hospitalization, health declines.
        - **Teeth and Mouth:** More likely to lose teeth to gum disease; proper care helps retain natural teeth.
        - **Digestion:** Sensitive to emotions (upset stomach if lonely/depressed). Constipation common (tissue/muscle changes, reduced thirst). Exercise, fiber, fluid help. **Dehydration** is serious problem (decreased thirst, reduced water conservation).
        - **Circulation:** Older heart slows, pumps less efficiently. Decreased circulation -> cold sensitivity, forgetfulness, poor cognition. Blood vessels lose elasticity -> blood pooling in feet/legs (edema). Increased risk of "little strokes" (TIAs). Pressure ulcers in immobile persons due to impaired circulation. Medications impact circulation.
        - **Cardiovascular and Respiratory Systems:** Heart muscle rigid, cells enlarge, arteries stiffen/accumulate plaque -> less forceful pumping, slower blood flow, less oxygen to tissues. Lung tissue loses elasticity, capacity reduced by half -> less oxygen absorbed, more out of breath. Exercise helps.
        - **Immune System:** T cells less effective. Auto-immune response (turns against body tissues). Increased risk of infectious diseases, CVD, cancers, rheumatoid arthritis, diabetes.
        - **Sleep:** Harder to come by, less, lighter, more trouble falling asleep. Men may have more problems (prostate enlargement). **Sleep apnea**

(breathing ceases). Restless legs can disrupt sleep. Poor sleep affects daytime energy, can lead to depression. Sleep aids can have rebound effects.

- **Sexuality:** Desires and physical capacity continue throughout life. Loss of interest usually due to emotional causes, drug use, or disease, not aging itself. Health and willing partner important.
- **Physical Disabilities:** Increase (CVD, cancer, respiratory diseases, stroke, osteoporosis, arthritis, adult-onset diabetes, unintentional injuries). Not caused by aging, but incidence increases.
  - **Primary aging:** Biological aging even in good health.
  - **Secondary aging:** Declines due to hereditary defects and negative environmental influences (poor diet, lack of exercise, disease, substance abuse, pollution, stress).
  - **Arthritis:** Inflamed/painful/stiff joints/muscles. Two forms: **Osteo-arthritis** (deteriorating cartilage) and **Rheumatoid arthritis** (autoimmune, whole body inflammation, deformed joints).
  - **Adult-onset diabetes:** Pancreas insulin output issue, damages blood vessels, increases risk of stroke/heart attack/circulatory problems, injury to eyes/kidneys/nerves.
  - **Unintentional injuries:** Death rate increases after 65. Car collisions (older drivers have higher rates of tickets, accidents, fatalities). Falls (30% over 65, 40% over 80) common, often lead to hip fractures and increased mortality. Many never regain ability to walk unassisted. Prevention includes corrective eyewear, improved home/car safety, family transport help.

Unit 2: Cognitive Changes (Early Adulthood, Middle Age, Old Age)

- **2.0 Introduction & 2.2 Early Adulthood**
  - Young adulthood (20-40) is a time to finish school, find a career, and create a family.
  - Physically healthiest, peak performance.
  - Cognitively, a time to **grow up and make life decisions**.
  - Erikson: **Intimacy vs. Isolation** stage (intimate relationship vs. isolation).
  - New sense of independence and responsibility.
- **2.2.1 Cognitive Changes in Early Adulthood**
  - Characterized by **realistic and pragmatic thinking**.
  - Also **reflective and relativistic thinking**.
  - **Piaget's view:** Young adults are **quantitatively advanced** (more knowledge) but qualitatively similar to adolescents. They increase knowledge in specific areas. Idealism of formal operational stage declines.
  - **Realistic thinking:** Looking at all aspects (positive, negative, neutral) before conclusions, balanced view of self/others/world.

- **Schaie's theory of development:** Adults progress beyond adolescents in their **use of intellect**, switching from acquiring knowledge to **applying it in everyday life**.
    1. **Achieving Stage:** Applying intelligence to situations with profound consequences (careers), mastering cognitive skills to monitor behavior, gaining independence.
    2. **Responsibility Stage:** Begins early adulthood, extends to middle adulthood. Focus on family needs (spouse, children), social responsibilities, career, responsibility for others at work/community.
- **Labouvie-Vief:** New integration of thought, less reliance on logical analysis, idealized logic replaced with commitment, focus on finding niche in workplace/society. Cognitive abilities remain strong.
- **William Perry's Reflective and Relativistic Thinking:**
    1. **Dualistic thinking (adolescents):** World seen in polar terms (right/wrong, good/bad).
    2. As youth mature, it's replaced by **multiple thinking**: Awareness of diversity of opinion, realizing authority may not have all answers, creating own thinking style, believing others are entitled to opinions.
    3. Leads to **relative subordinate thinking**: Personal opinions challenged, logical evaluation of knowledge pursued.
    4. Culminates in **full relativism**: Complete understanding that truth is relative, knowledge is constructed, contextual, not absolute.
- **Postformal thought:** Qualitatively different from Piaget's formal operational. Involves reflective thinking, answers varying by situation, ongoing search for truth, realistic solutions, and influence of emotion/subjective factors on thinking.

- **2.3 Middle Adulthood (40-60 years)**

    - Period between young adulthood and old age.
    - Challenge: develop concern for future generations, contribute through family/work.
    - Characterized by **competence, maturity, responsibility, and stability**.
    - Time to enjoy job success, family, social life, children's success.
    - Increased focus on health, children's fate, aging parents, leisure, plans for old age.
    - Midlife crisis: time of re-examination and re-evaluation of lives, relationships, work, meaning.
    - Often settled in careers; job satisfaction and commitment tend to be high.
    - Adjust idealistic hopes to realistic possibilities, retirement planning.
    - Can lead to stress/sadness over unaccomplished goals, sometimes midlife career change.
    - Financial adjustments: planning for college for children; women may enter/reenter workforce.
    - Leisure important for well-being and dealing with stress.
    - Period of creativity and significant contribution to society.

- **2.3.1 Cognitive Changes in Middle Adulthood**
    - Cognitive development is **multidirectional**: gains in some areas, losses in others.
    - Cross-sectional studies show decreases in intelligence with age; longitudinal studies show increases until 50s.
    - **Intelligence: Crystallized vs. Fluid**
        1. **Fluid intelligence:** Ability to see relationships, use abstract reasoning, analyze information, solve novel problems. Primarily genetic. Peaks during early adulthood, then **declines with age** (e.g., abstract thinking, speed of thinking). Best works for mathematicians, scientists, poets in 20s/30s.
        2. **Crystallized intelligence:** Ability to use knowledge, experience, vocabulary, verbal memory. Reflects accumulated facts, information, knowledge from education and culture. **Improves through middle age and beyond**. Allows best works for historians, philosophers, prose writers in 40s/50s and older. Individuals use stored information automatically.
        3. Fluid intelligence decline is temporarily masked by increase in crystallized intelligence.
    - **IQ tests** may lack ecological validity due to slowing reaction time.
    - **Robert Sternberg's Three Parts of Intelligence:**
        1. **Analytic/Academic:** Mental processes for efficient learning, remembering, thinking. Rewards one right answer tests (e.g., multiple choice). Associated with extensive, highly organized knowledge, increased work satisfaction, commitment, well-being.
        2. **Creative:** Capacity to be **flexible and innovative** in new situations. Peaks during middle adulthood (e.g., writing). Expertise increases; use accumulated experience to solve problems; more creativity/flexibility than novices. Make own decisions, resourceful, imaginative use of words, originality.
        3. **Practical:** Enables adaptation to contextual demands. Associated with pleasant time after work, more time/money for activities, decreased heart disease/death due to leisure. Preparation for retirement.
    - **Information Processing and Memory:** Speed of information processing, reaction time, and memory **decline**. Effective memory strategies can decrease this decline.
    - **Religion, Health, Coping, and Meaning in Life:**
        1. Significant increase in **religiosity and spirituality**.
        2. Positive association of religious participation and longevity.
        3. Religion promotes physical/psychological health and positive coping.
        4. Many middle-aged individuals increasingly examine **life's meaning**.
- **2.4 Old Age (65+ years)**

- Erikson: important to find **meaning and satisfaction (integrity)** rather than bitterness and disillusionment (**despair**).
- Not necessarily synonymous with substantial mental or physical deterioration. Many older people are happy and engaged.
- **Gerontology:** Interdisciplinary field studying aging and the aging population.
- Theories of successful aging:
    1. **Disengagement theory:** Withdrawal from society is normal/desirable, relieves responsibilities, opens opportunities for younger people.
    2. **Activity theory:** Activity necessary to maintain "life of quality" ("use it or lose it"), better adjustment to aging if active physically, mentally, socially.
- **Ageism:** Prejudice or discrimination based on age, often targeting older people (e.g., forced retirement, stereotyping).

- **2.4.1 Cognitive Changes in Old Age**

    - General cognitive loss closer to end of life.
    - **Response speeds (neural and motor) decline**.
    - Age-related decrease in **working memory** is crucial for poorer cognitive performance.
    - **Selective optimization with compensation:** Means of making best use of cognitive skills (narrow goals, select valued activities, compensate for losses).
    - **Factors related to cognitive change:** Mentally active people maintain abilities. Retirement can impact cognitive abilities depending on how time is used.
    - **Terminal decline:** Steady, marked decrease in cognitive functioning prior to death.
    - **Memory:**
        1. Older adults take in information slower, use strategies less, can't inhibit irrelevant info, retrieve info from long-term memory less effectively. Memory failure increases. Slower processing speed -> less retained from current activities. Forgetting context.
        2. **Recognition memory** does not decline as much as **free recall**.
        3. **Implicit memory** (without conscious awareness) is more intact than deliberate memory.
        4. **Associative memory deficit:** Problem creating/retrieving links between info, more common for elders.
        5. **Remote memory:** Very long-term recall; not clearer than recent recall (myth dispelled).
        6. **Autobiographical memory:** Best recall of adolescent and early adulthood experiences (novelty, life choices, emotional charge).
        7. **Prospective memory:** Remembering planned future activities; more forgetfulness/absentmindedness.
    - **Language Processing:** Two aspects diminish: finding right words, planning what/how to say. Speech may have more pronouns, unclear references, slower, more pauses, hesitations, false starts, word repetitions. Tend to simplify grammatical structures.

- **Problem Solving:** Declines in late adulthood. Married people collaborate more. Better at solving problems they feel are under their control (e.g., health).
- **Wisdom: Increases with age**. Includes practical knowledge, ability to reflect/apply, emotional maturity, listening skills, creativity to help others. Develops from dealing with life difficulties and adapting to change. Associated with better education and physical health. Requires insight into human condition, often found in leadership roles.
    1. Components: knowledge of life concerns, effective strategies, multi-demands perspective, concern for human values/common good, awareness/management of life uncertainties.
- **Cognitive Interventions:**
    1. **Lifelong learning:** Programs like Elderhostel, university classes for seniors.
    2. **Benefits:** Learning new information/ideas, making friends, broader perspective, challenging stereotypes, valuing diversity.
- **Retirement:** Conventional choice at 65. Happier if not forced to retire and have adequate income.
- **Widowhood:** Women tend to outlive men; very stressful if spouse dies early. Support groups/counseling can help.
- **Death and Dying (Kübler-Ross):** Five basic reactions for terminally ill patients:
    1. **Denial:** Deny reality, isolate self.
    2. **Anger:** Envy the living, "Why me?".
    3. **Bargaining:** Plead for more time.
    4. **Depression:** Recognition of inevitability, separation from family, exhaustion, futility.
    5. **Acceptance:** If death not sudden, peace with inevitable.
- **Hospice:** Hospital for terminally ill, aims to maintain good quality of life for patient/family.
- After death of loved one: shock -> grief -> apathy/depression. Support groups help.

Unit 3: Psychosocial Changes (Early Adulthood, Middle Age, Old Age)

- **3.0 Introduction & 3.2 Development During Adult Years**

    - Growth stabilizes, individual looks forward to **concrete roles** (family, marriage, career).
    - Capabilities and behaviors change due to biological growth and interaction with environment.
    - Indian tradition: **Grihastha ashram** (social life, family responsibilities, marriage, career).
    - Adult years have sub-periods: young, middle, late adulthood.
    - **Midlife crisis:** Reevaluation leading to questioning beliefs/values. May result in divorce, job change, moving. Often response to sense of mortality. Not everyone experiences a crisis, some have a **midlife transition**.

- **Normative crisis model:** Psychosocial tasks occur in age-related sequence (e.g., midlife transition 40-45).
- **Timing of events model:** Tasks occur in response to specific life events and their timing (e.g., questioning life desires at any age).
- Late adulthood (60+): Most lose jobs (retire). Fear physical/psychological health.
- Elderly often perceived as inactive, intellectually deteriorating, narrow-minded, religious. However, many remain healthy and active (lifestyle: exercise, diet, check-ups).
- Grandparenting helps satisfy personal/emotional needs, role models, self-fulfilling.
- **Successful aging:** Elders minimize losses and maximize gains.

- **3.2.1 Stage Theories Versus the Contextual Approach**

  - **Stage Theories:**
    - All humans move through an **orderly progression of stages** in development, regardless of where/when they live.
    - Individual grows in a **systematic process**.
    - **Erik Erikson's Eight Stages of Life:** Development proceeds through distinct stages, each with a **psychosocial crisis** resulting from biological drives and societal demands.
    - **Adult Crises according to Erikson:**
      - **Intimacy vs. Isolation (Late Adolescence/Early Adulthood):** Develop ability to form **deep, intimate relationships** (strong emotional attachment, not just sexual). Capacity to love deeply and consistently. Failure to resolve leads to isolation, inability to form lasting relationships. Intimacy requires giving up some independence. Successful: cooperative, tolerant, accepting of differences, comfortable with aloneness. Isolation: competitive, threatened by closeness, not accepting differences.
      - **Generativity vs. Self-Absorption (Middle Adulthood):** Need to **overcome selfish concerns** and take active interest in **helping/guiding the next generation**. For parents: children. After children grown: mentor younger generation (students, co-workers, nieces/nephews). Failure to resolve (stagnation): absorbed in own lives, cut off from growth/satisfaction. Generativity includes all things people generate that can outlive them and improve society (parenting, mentoring, volunteering, community involvement, creative endeavors). Generative people are better adjusted, low in anxiety/depression, high in self-acceptance/life satisfaction.
      - **Integrity vs. Despair (Final Decades/Old Age):** Look back and analyze life's meaning. "Did my life have meaning?" "Did I matter?". Yes -> sense of **integrity** (whole, complete, satisfied, accepted setbacks/disappointments, celebrated successes, found meaning). No -> intense feelings of **despair** (wrong decisions, too

late to remedy). Successful resolution impacts acceptance of mortality and health.

- **Contextual Approach:**
  - Takes into account **differences in one's life and situations**.
  - The **context or environment** is more important than inbuilt mechanisms.
  - Individuals develop according to **environmental and contextual influences**.
  - Biological influences are seen as secondary or ignored.
  - Key theorists: **Urie Bronfenbrenner** (Ecological Systems Theory) and Lev Vygotsky.
  - **Bronfenbrenner's Ecological Systems Theory:** Focuses on changing relations between individuals and environments.
    - **Five environmental systems:** Microsystem, Mesosystem, Exosystem, Macrosystem, Chronosystem.
    - **Microsystem:** Immediate environment where individual lives and directly interacts (family, peers, school, neighborhood).
    - **Mesosystem:** Linkages between microsystems.
  - **Active Role of Individual:** Developing individual is not passive but actively influences their environment.

- **3.3 Psychosocial Changes During Early Adulthood**

  - Time for **self-search and preparation** for future.
  - New sense of **independence and freedom**, with added personal responsibility.
  - Learning about self and others through social interaction.

- **3.3.2 Levinson's Seasons of Life Theory**

  - Sought common path of change in adulthood.
  - Believed in stages with inherent tasks, each starting with a 5-year transition.
  - Stable periods (5-7 years) where a person builds a **life structure**.
  - **Life structure:** Underlying design of a person's life, involving relationships with significant others and occupations; harmonizes inner and outer demands.
  - Early adulthood: greatest energy, contradiction, stress, but also intense satisfaction as one charts their own course.
  - **Dreams and Mentors:** Most construct a "dream" (image of self in adult world) during early adult transition (17-22), guiding decisions. Men's dreams more individualistic (business/career success); women's related to supporting men's goals.
  - **Age-30 Transition:** Time to reevaluate life structure. Singles may look for partners. Women in marriage/child-bearing may assert individual goals. Can be a crisis if relationships/occupation unsuccessful.
  - **Settling Down (Men):** Focus on certain relationships/aspirations, establish a niche consistent with values (family, wealth, power, achievement).
  - **Continued Instability (Women):** Often sidetracked by child-bearing/family; attain stability later than men.

- **The Social Clock:** Age-graded expectations for life events (first job, marriage, children, house, retirement).
    - Women following a **feminine social clock** (marriage/child-bearing in 20s) -> responsible, self-controlled, tolerant, caring; but may experience self-esteem decline, feel vulnerable with age.
    - Women following a **masculine social clock** (early career development) -> more dominant, sociable, independent, intellectually effective.
    - Women not following a social clock may suffer from self-doubt, incompetence, loneliness.

- **3.3.3 Attachment Patterns and Romantic Relationships**

    - **Close Relationships:** Require finding a partner, building emotional bonds, and true intimacy.
    - **Romantic Love:** Finding a partner to share life/dreams, enhances self-concept/well-being. Mate selection often involves similar background, age, ethnicity, SES, religion, and physical proximity.
    - **Sternberg's Triangular Theory of Love:**
        - **Intimacy:** Emotions of tenderness, warmth, concern for others, desire for reciprocity.
        - **Passion:** Sexual component, romance, physical arousal. **Passionate love** is strong at relationship beginning but declines as partner becomes known.
        - **Commitment:** Determines if a relationship will survive. Requires warmth, forgiveness, sensitivity, acceptance, respect.
        - **Companionate love:** Formed with intimacy and commitment; partners develop warm, trusting affection and caregiving. Holds long-term relationships together.
    - **Attachment Patterns & Romantic Relationships:** Early attachment patterns predict quality of later intimate relationships. Early bond sets up an **internal working model** (expectations about love figures).
        - **Secure Attachment:** View themselves as likable, open to others, comfortable with intimacy, few fears of abandonment. Relationships are trusting, happy, partner as friend; willing to seek comfort, satisfying sexual behavior.
        - **Avoidant Attachment:** History of demanding/disrespectful/critical parents. Internal models emphasize independence, mistrust partners, anxious about closeness. Believe others dislike them, true love doesn't last. Relationships characterized by jealousy, emotional distance, little enjoyment of physical contact. May become workaholics or engage in affairs.
        - **Resistant Attachment:** Unpredictable/unfair parents. Set up intense relationships with fears of abandonment and smothering partner. Experience extreme highs and lows, poor boundaries, inappropriate disclosure.

        - Partner's internal security fosters security in the other. Healthy relationships can help overcome troubled backgrounds.
    - **Friendships:** Usually similar in background, age, interests, needs. Offer affirmation, acceptance, support. Sharing deep feelings can be more open in friendship than marriage, especially for women.
        - **Same-sex friendships:** More intimate for women (talking); men do activities (sports), may feel competitive and resistant to revealing vulnerabilities.
        - **Other-sex friendships:** Less often, don't last as long. Men and women disclose more to women. Women can learn from male friends about male motivations and objectivity.
        - **Siblings as friends:** Especially sisters, become companions in adulthood. Rivalries subside, supportive relationships develop. Close sibling relationships predict mental health.
- **3.4 The Family Life Cycle**
    - Sequence of phases in family development: living alone, marriage, children, children leave, retirement, loss of spouse.
    - **Leaving Home:** Earlier ages now (school, military, jobs). Youth in chaotic/overly dependent families may leave earlier. Most can return if financial setbacks.
    - **Joining of families in marriage:** People waiting longer to marry (M: 25, W: 27). Represents connection between two family systems with traditions/expectations.
    - **Marital roles:** Meals, time together/apart, leisure, sexual needs, financial decisions. Communication and negotiation skills are crucial.
    - **Traditional marriages:** Clear division of labor (man: economic, woman: home/children).
    - **Egalitarian marriages:** Spouses relate as equals, share power/authority, balance work/family. Division of labor in home may still not be equal.
    - **Marital satisfaction:** Men report being happier. Marriage enhances men's health. Women need certain relationship quality; suffer more when problems. Women may suffer under multiple role demands. Women willing to work through discussion, men may withdraw.
    - **Parenthood:** Active choice due to birth control. Less stigmatization for childless. Decision based on financial circumstances, religious values, health. Traditional identities often choose children; high-status/demanding jobs less often.
        - **Advantages of children:** Warmth, affection, fun, learning experiences, passing on values/family name, accomplishment.
        - **Disadvantages:** Loss of freedom, financial burdens, being tied down, conflict between work/family.
    - **Transition to Parenthood:** Loss of sleep, new tasks/responsibilities, less couple time, increased expenses. Roles become more traditional, even for egalitarian couples, causing frustration. Loving, supportive marriages manage strain; troubled marriages become more distressed. Husband's negativity predicts drop in mother's satisfaction. Interventions can get fathers more involved.

- **Career Development:**
    - Aids in establishing satisfying identity (skills, accomplishments, friends, financial independence/security).
    - **Establishing a career:** Men enter earlier, continuously. Promotion linked to job commitment, competitiveness. Successful men emphasize jobs over family/leisure. Self-efficacy important; fear of failure leads to lower goals.
    - **Combining Work and Family:** Dual-career marriages are challenging, especially for women due to **role overload**. Role overload linked to stress, poorer marital/parenting relationships, child behavior problems. More problematic for lower-paying jobs with fewer options. Professional careers more flexible, but moves difficult for dual careers.
- **3.5 Psychosocial Changes During Middle Adulthood**
    - **Erikson's Theory:** Primary psychosocial task is to develop **generativity** (desire to expand influence/commitment to family, society, future generations). Midlife adult concerned with forming and guiding next generation. Failure leads to **stagnation** (self-absorption, self-indulgence, invalidism).
    - **Generativity:** Reaching out to others to give and guide. Extends beyond nuclear family to community/social group. People try to integrate goals with welfare of larger world. Includes anything generated that can outlive them and improve society (parenting, mentoring, volunteering, creative endeavors). People need to feel needed, like their lives amount to something eternal.
    - **Stagnation (Alternative to Generativity):** Becoming self-centered and narcissistic. Place own comfort/security above challenges involving others. Detachment from children, self-centered focus on what they can get, losing interest in productivity/talents.
    - **Levinson's Seasons of Life (Middle Adulthood):** Begins with a transition (40-45), followed by a life structure (45-50), re-evaluated (50-55), culminating in a final life structure (55-60).
    - **Midlife Transition:** Occurs around age 40, evaluation of success in meeting adult goals. Realize more time behind them than ahead. May lead to big changes in family/occupation. May turn inward, focus on own needs.
- **3.5.1 Four Developmental Tasks of Middle Adulthood (Levinson-like)**
    - **Young-Old:** Seeking new ways to be young and old, giving up inappropriate youthful qualities, finding positive meaning in changes.
    - **Destruction-Creation:** Re-evaluating past hurtful acts, attempts to apologize/make amends, general efforts to be kinder/more creative.
    - **Masculinity-Femininity:** Recognizing and integrating masculine/feminine aspects of personality (men more empathic/caring, women more autonomous/assertive).

    - **Engagement-Separateness:** Finding a better balance between engagement with outside world and interior needs. Men may pull back from ambition; women may shift interests from family to job/community.
    - **Modifying the Life Structure:** People try to balance youth and age. Women fear aging due to societal emphasis on youth/looks. Integrating masculine/feminine characteristics (androgyny) is adaptive. Social context and supports (or lack thereof) influence this process.
    - **Midlife Crisis:** Controversy over whether it's a distinct stage. Changes often slow/steady. May occur around early 40s for men, late 40s/50s for women (due to freedom from child responsibilities). Many describe troubling moments prompting new goals/changes.
- **Stability and Change in Self-Concept and Personality:**
    - **Possible Selves:** Ideas of what one can become or fears becoming. In 20s: varied, positive, idealistic. With age: more realistic, fewer, more modest, related to competence in selected areas. Future seems less open-ended.
    - **Self-Acceptance, Autonomy, and Environmental Mastery:** People become more introspective, change lifestyles to suit needs.
    - **Coping Strategies:** Finding "silver lining," planning, evaluating alternatives, using humor.
    - **Gender Identity Shifts:** In response to shifting sex hormones. Women adopt more masculine traits (assertiveness, self-confidence); men become more sensitive, caring, considerate. People become more androgynous.
- **Relationships at Midlife:**
    - Called **"launching children and moving on"**. Not seen negatively, even for women. Can last up to 20 years before retirement.
    - Establishes different relationships with children, new relationships with in-laws and grandchildren. Also cope with elderly parents' needs.
    - **Marriage and Divorce:** Households are economically well-off. Divorce may be chosen at this time, more manageable emotionally.
    - **Feminization of poverty:** Trend where self-supporting women/families become majority of adult poverty population.
    - **Causes of Divorce:** Poor communication, substance abuse, physical/verbal abuse, desire for autonomy. Women who survive divorce tend to be more tolerant, comfortable with uncertainty, nonconforming, self-reliant.
    - **Changing Parent-child Relationships:** More positive when parents have own activities and relinquish authority. Authoritative parenting links to better contact after child leaves and better parent satisfaction. Parents often feel they give more than they receive (financial help, pet care). Value time together.
    - **Grandparenthood:** A stage with fewer responsibilities, mostly fun. Meanings: valued elder, immortality through descendants, reinvolvement with personal past, indulgence. Relationship depends on child's age (play to warmth/support/advice). Geographic proximity helps.

    - **Siblings:** Declining contact in midlife but increases with life events (weddings, divorce, babies, parents' illnesses). Sisters' relationships usually closer. Resentment if one sibling bears sole responsibility for elder care.
    - **Friendships:** Still cleave across genders (men with men, women with women). Women have more close friends and give/get more emotional support. Number of friends declines (more selective). Close friendships maintained with care; sources of emotional support, pleasure, well-being.
- **Career Development (Middle Adulthood):**
    - Job training less available to older workers; supervisors may have ageist stereotypes.
    - **Glass Ceiling:** Unspoken, invisible barrier to advancement for women and ethnic minorities. Women managers are effective, inspiring, considerate; team building suits female skills.
    - **Career Change at Midlife:** Usually to related work, sometimes for more stimulating/relaxing work. Drastic shifts often signal personal crisis.
    - **Unemployment:** Serious problem for middle-aged workers (less employable, lower salary); affects self-worth.
    - **Planning for Retirement:** Often inadequate; illness or crisis can short-circuit plans. Developing hobbies/community interests before retirement is crucial to avoid boredom/depression. Relocation needs careful thought.
- **3.6 Psychosocial Changes During Old Age**
    - Social world is varied; high value on friends, sometimes more than family. May avoid burdening family. Strive for independence.
    - Married persons tend to be happier than single; never-married cope best with loneliness.
    - **Erikson's Theory: Ego Integrity vs. Despair** (discussed above). Sense of wholeness, satisfaction, acceptance of setbacks. Peace about mortality. Despair is bitterness, hopelessness, anxiety about death.
    - **Peck's Theory: Three Tasks of Ego Integrity:**
        - **Ego differentiation vs. work-role preoccupation:** Finding self-worth beyond career after retirement.
        - **Body transcendence vs. body preoccupation:** Transcending physical limitations, finding value in cognitive/social strengths and relationships.
        - **Ego transcendence vs. ego preoccupation:** Facing death constructively, giving back to younger generation.
    - **Labouvie-Vief's Theory: Emotional Expertise:** Elders are more in touch with their feelings and use them to reflect on life experiences. Emotional sensitivity allows detachment from experiences to choose responses better. Better emotional self-regulation than younger people.
    - **Reminiscence and Life Review:**
        - **Reminiscence:** Telling stories about past people/events, thoughts/feelings.

        - **Life review:** Reminiscence reflecting on past experiences' meaning for self-understanding.
    - **Stability and Change in Self-Concept and Personality:**
        - **Secure and Multifaceted Self-Concept:** After a lifetime of self-knowledge, feel more secure, self-concepts become complex/multi-faceted.
        - **Agreeableness, Sociability, Acceptance of Change:** More flexible and optimistic approach. Agreeableness (generosity, good-naturedness) is higher. Sociability drops (selective relationships, loss of people). Acceptance of change linked to well-being.
        - **Spirituality and Religiosity:** Sense of spirituality encompasses lives more meaningfully, sense of truth/beauty in art/nature/relationships. Religion offers stabilizing rituals. Spirituality advances to a more reflective approach, at ease with unknowable aspects of life.
    - **Individual Differences in Psychological Well-Being (Control vs. Dependency):**
        - **Dependency-support script:** Dependent behaviors immediately attended to, reinforcing dependency.
        - **Independence-ignore script:** Independent behaviors ignored, also reinforcing dependency.
        - Being dependent is not a happy state; helper often resented. Lovingly offered help is easier to accept.
    - **Elder Suicide:** Not always evident; may involve quitting eating, self-care, meds, relationships.
    - **Health:** Strong predictor of well-being. Illness/chronic disabilities lead to loss of control, helplessness, social isolation. Mental health decline affects physical health.
    - **Negative Life Changes:** More frequent (loss of friends/spouses, poor health, financial strain, dependency). Women often hit more, but retain identity as others depend on them.
    - **Social Support and Social Interaction:** Reduces stress, promotes health/well-being, increases longevity. Distress if unable to reciprocate assistance.
- **3.6.2 Relationships in Late Adulthood**
    - **Social Convoy:** Cluster of family members and friends providing safety and support. Bonds change, some closer]. Number of friends declines (more selective). Close friendships maintained with care.
- **3.6.3 Retirement and Leisure**
    - **Decision to retire:** Depends on affordability, health, meaningful activities, early benefits, gender, ethnicity. Women often retire earlier due to family demands.

- **Adjustment to Retirement:** Affected by health status, financial stability, sense of personal control over decision, characteristics/satisfaction from work, social support, marital happiness.
- **Leisure Activities:** Relate to physical/mental health, reduced mortality. Best to develop hobbies/interests/volunteer activities before retirement.
- **Successful Aging:** Occurs when elders minimize losses and maximize gains. Fostered by social contexts: well-funded social security, good health care, safe/adapted housing, social services, lifelong learning, in-home help, sensitive nursing home care.

Unit 4: Challenges and Issues in Ageing Process

- **4.0 Introduction & 4.2 Ageing Process**
  - Aging (or ageing) is a process that **accumulates changes in organisms over time**.
  - Human aging involves **multidimensional changes: physical, psychological, cultural, and social**.
  - Process begins at birth and continues throughout life.
  - Body constantly changes (e.g., cell development, shedding, hair/skin adaptation).
  - "Normal changes" include gray hair, wrinkles, hearing loss, need for bifocals, hardening of arteries, bone density loss, slowing metabolism. These are not deterioration but normal progression.
  - **Aging retardation:** Concept of extending lifespan.
  - Aging process is a **complex biochemical process**, inevitable, with signs both internal and external.
  - Intensity of problems depends on **genetic and environmental factors**. Genetic factors seem more powerful in determining rate of aging.
  - **Heredity, diet, exercise, social involvement, and spirituality** play a large part in physical well-being. Eating right and exercising can stave off or slow aging.
- **4.2.1 Ageing Process in Women**
  - Women are often concerned with their looks; physical signs of aging can be shattering.
  - More sensitive gender, emotional and psychological changes are difficult to cope with.
  - **Physical Challenges:**
    - **Menopause** is key: hormonal changes cause problems.
    - Loss of **bone mass and muscle strength** leads to osteoporosis and arthritis.
    - Skin becomes sunken, sallow, develops wrinkles.
    - Weakened immune system makes body susceptible to illnesses.
    - Most vital organs show diminished performance.
  - **Psychological Challenges:**

        - Related to physical status; hormonal changes during menopause bring behavioral changes.
        - Loss of fertility can lead to **depression**.
        - **Emotional insecurity** due to fear of losing physical attractiveness, feeling of burden, stress from simultaneous changes, and relating aging to "the end".
        - Optimistic women may take it as an opportunity to fulfill dreams.
    - **Social Challenges:**
        - Depends on attitude and health.
        - Poor health may lead to social withdrawal.
        - Outgoing women may engage in community activities or hobbies.
        - Often financially settled with independent families, providing a "golden chance" to live for themselves.
        - Key to fighting aging ishigh fiber, low saturated fat, rich in nutrients), consistent moderate exercise, stress management are foundations for health.
    - **Mental Health:** Concerns about **Alzheimer's Disease and Dementia**. Maintaining function: herbal extracts (ginkgo biloba), nutrients (phosphatidyl serine, DHA). Ginkgo helps circulation to brain/extremities, acts as antioxidant, protects nerve cells.
    - **Heart Health:** Most obvious concern. Cardiovascular disease is a leading cause of death. Diet, lifestyle, natural medicines play significant role in prevention and treatment.
    - **Prostate Health:** A consideration for men after age 40. 60% of men over 40 have an **enlarged prostate (BPH - benign prostatic hyperplasia)**, likelihood increases with age. Proactive care reduces serious complications. Herbs like saw palmetto, nettle, pygeum, pollen extracts help. Reduce effects of testosterone breakdown products (DHT linked to prostate cancer).
    - **Skeletal Health:** Joint and bone health issues are common (osteoarthritis from past injuries/wear and tear). Connective tissue repairs less efficiently. Nutrition needed for cartilage healing. **Osteoporosis** is a major concern for men too (1/6 men will fracture a hip).
    - Aging is natural; doesn't mean loss of function/vitality. Healthy habits can make 40+ years the best.
- **4.3 Ageing Issues and Challenges in Early Adulthood**

    - (Definition already covered above: 20-40 years, vibrant, healthy, focus on friendship, romance, child-bearing, careers).
    - **Intimacy and Individual Development:** Cannot exist alone. Child's birth initiates lifelong mutual adaptation process. Intimate interactions affect adaptations to changing needs/stresses. Early intimate interactions form basis for later relationships. Adapting to others' styles requires adjusting one's own behaviors.
    - **Maturity:** Adolescents adapt to adult world, come to terms with emerging self (self for sameness and continuity. Importance of intimate friendship/romance

stems from valuable dialogues in adolescence. Personality differences identified by capacity to form intimate relationships (commitment, depth, partner individuation) based on early life interactions.

- **4.4 Ageing Issues and Challenges in Middle Adulthood**
    - (Definition already covered: midlife transition, period beyond young adulthood, before old age).
    - **Physical Appearance:** Hair greying/whitening, teeth yellowing, more wrinkles, sagging skin.
    - **Hearing and Vision:** Hearing loss (especially high-pitched noises) appears around 40. Men experience twice as much hearing loss as women. Vision decreases (focus declines, need brighter lights).
    - **Health:** Fewer infectious diseases, more **chronic illnesses**. **Heart disease and cancer** are common. Men are more prone to heart disease, cancer, stroke (especially if divorced). Women have more non-fatal illnesses (goiter, gout, arthritis2].
    - **Relationships during Middle Adulthood:**
        - Primary long-term relationships: **cohabitation and marriage**. Most adults are married.
        - **Marital Satisfaction (U-curve):** Happiest in early years, less happy in middle years, then increases again later (finances stable, parenting responsibilities end).
        - **Divorce:** Reasons varied (extended crisis, growing in different directions, incompatibility). Both parties usually responsible.
        - **Love Changes:** Passion fades as intimacy and commitment build. Many experience **companionate love** (committed and intimate, but not passionate). Can improve communication, rekindle passion, grow together. Understanding evolves over time.
        - **Relationships that last:** Long-term commitment, verbal/physical appreciation, emotional support, considering partner best friend. Effective communication is essential for intimacy.
        - **Friends:** Offer support, direction, guidance, change of pace. Fewer friends due to high life responsibilities, but quality predominates.
        - **Children:** More middle adults rearing small children due to later marriage/parenthood. Middle adults and adolescents prone to simultaneous emotional crises (generativity vs. identity). Parents may try to live out fantasies through children or make them "improved versions". Children nearing adulthood can trigger a midlife crisis for parents.
        - **Relationships with Grown Children:** Typically maintain close relationships, often feel they give more than receive (financial help, pet care) [4
- **4.5 Ageing Issues and Challenges in Late Adulthood**

- **Elder care:** Umbrella of services (meals, socialization, personal care, housekeeping, residential facilities, adult day care).
- Physical differences among peers increase with age (sum of life experiences and habits).
- Chronological age has little bearing on biological age.
- **Body Changes:** Muscles, blood vessels, tissues lose elasticity. Heart less efficient, bones weaker, metabolism slows. 425]. Recommendations: reduce calorie/alcohol intake, increase healthy fats, avoid trans fats/high-fructose corn syrup.
- **Arthritis:** Affects nearly half elderly, leading cause of disability. Prevention: avoid overuse, regular exercise, weight management.
- **Osteoporosis and Falls:** Affects most adults 50+ (mostly women), risk increases with age. Prevention: calcium, vitamin D, weight-bearing exercise.
- **Cancer:** Risk increases with age. Cervical cancer decreases, endometrial cancer increases. Prostate cancer increases with age (higher in Black men). Lung cancer accounts for more deaths than breast, prostate, and colon cancers combined.
- **Vision and Hearing Loss:**
    - Age-related eye diseases (macular degeneration, cataract, diabetic retinopathy, glaucoma) affect people 40+. Antioxidants helpful for macular degeneration. Regular eye exams for glaucoma (can be arrested but vision loss not restored).
    -].
    - **Retirement:** A big life change; sense of worth tied to work. Depression and suicide rates can rise.